

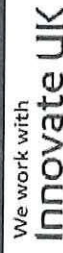


Earlier “early intervention”: a novel & evidence-based approach

*Promoting early return to work self-management
in employees on sickness absence*

Stephanie McCahon, Swiss Re
Prof Kim Burton, University of Huddersfield
Dr Abasiama Etuknwa, University of Huddersfield

11 July 2022 – Health Claims Forum



Stephanie McCahon,
Claims Medical Manager, Swiss Re

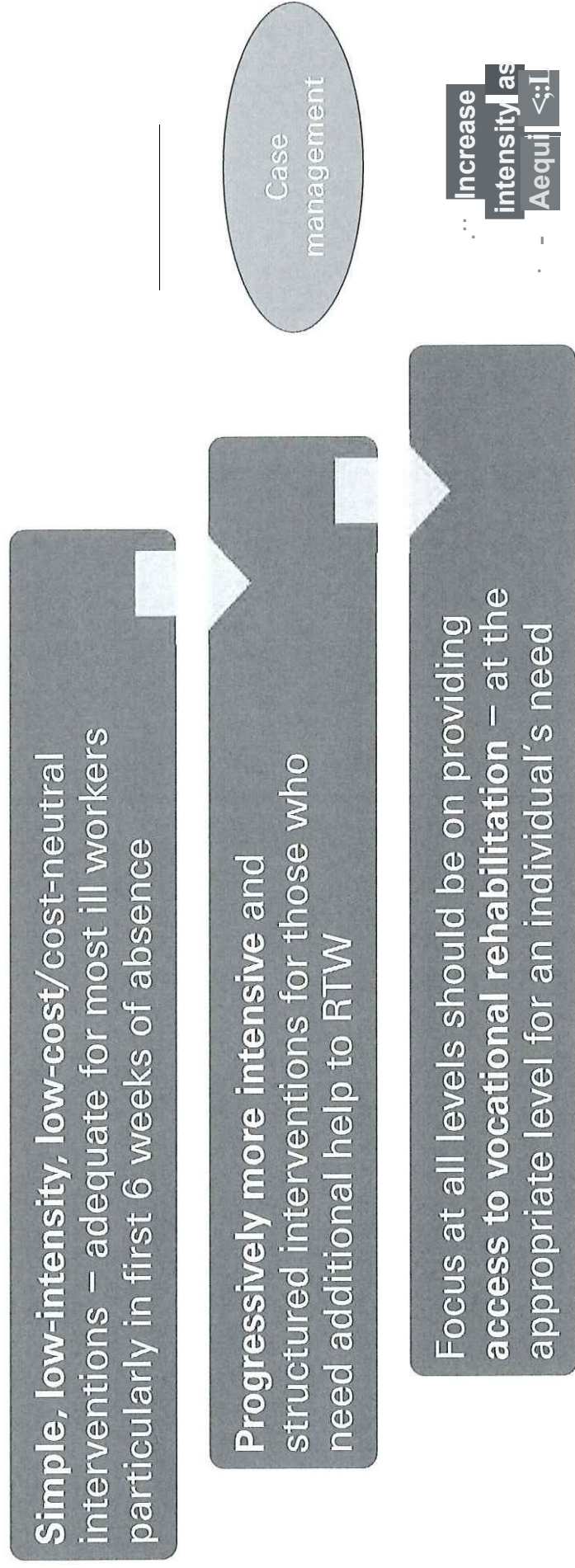
The background



We help our clients
to help people
back to work
and better
health



What works, for whom, and when: a *stepped-care* approach



Prof Kim Burton,
University of Huddersfield

The science



Health Claims Forum 2022

Kim Burton, OBE, PHO, HonFFOM

University of
HUDDERSFIELD
Inspiring global professionals



Work is what defines us:

" and what do you do?"

The WORK () HEALTHdouble act

I see my daddy walking
through them factory gates
in the rain

**Factory takes his hearing,
factory gives him life**

Bruce Springsteen, 1978



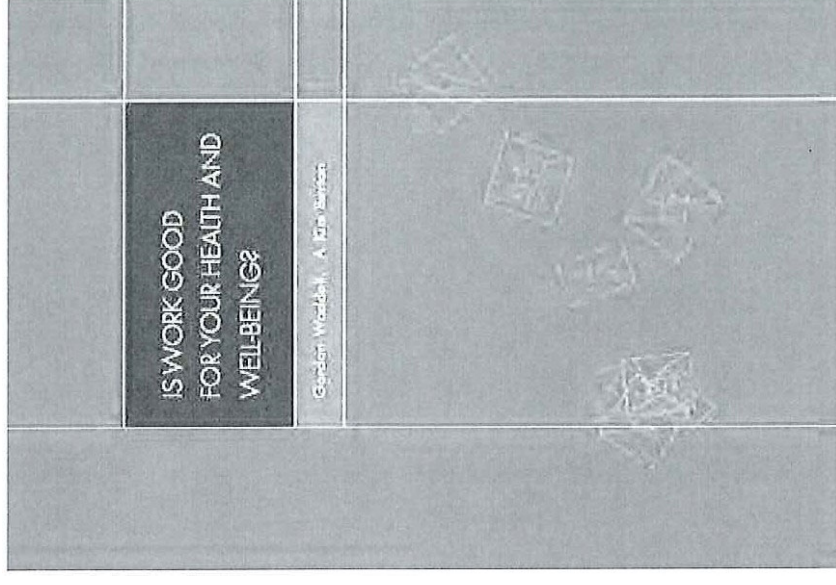
"Work is life, you know,
and without it, there's
nothing but fear and
insecurity."

John Lennon, 1969

..but is work actually good for us?

Is it good for us?

- Emphatic YES - work is health promoting
 - with the proviso that it's good work that's good
 - certainly, worklessness is decidedly bad
- Importantly, getting back to work reverses the adverse health effects of worklessness
- So, facilitating people's work ability is good public health policy
- Trouble is, we're not doing it too well

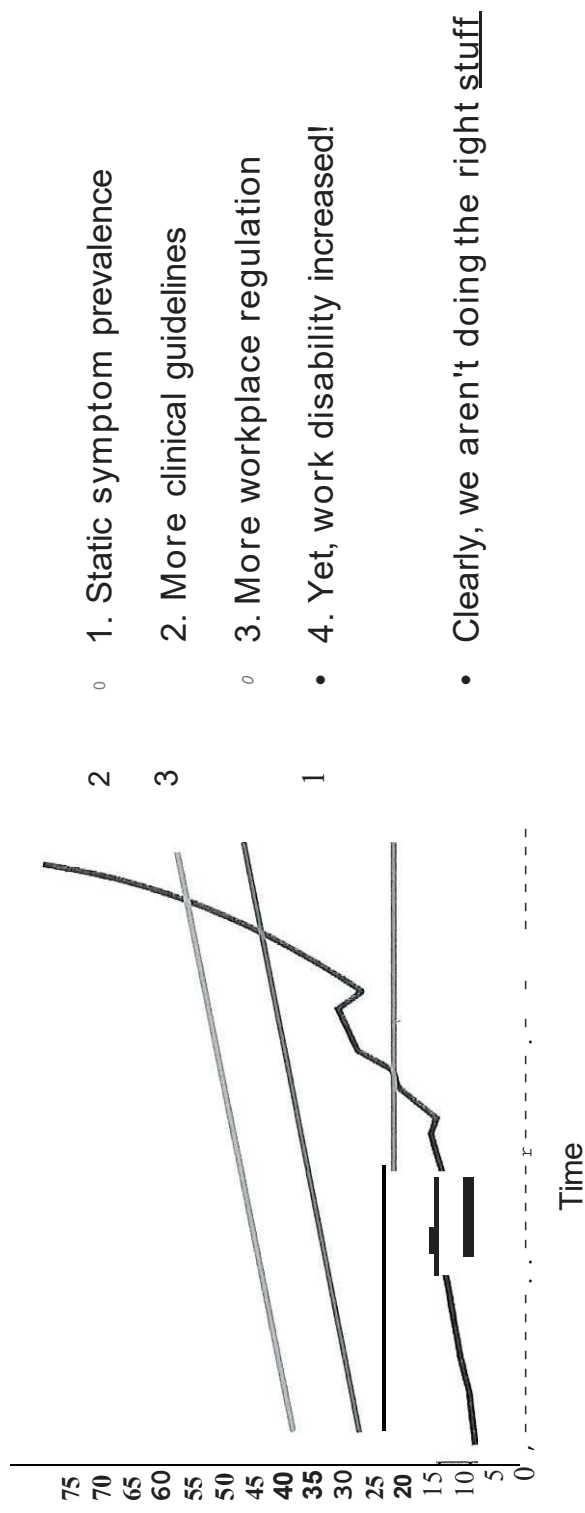


2006

Needless work disability

- the story starts with a paradox

4



Time for change

Actually, there have been quite a lot of UK policy initiatives over the last decade - but:

change seemingly takes forever

"Nothing happens, and it keeps not happening, forever"

(Prof Brian Cox)

Changing policy is beset with all manner of difficulties, so sadly...



The slide to disaster

social constructs+ separated systems

continues

increasing obstacles

- Before symptoms
- At onset of symptoms
- Workplace not helpful
- On seeking healthcare/diagnosis
- If signed off work
- If no vocational rehab
- If work disability persists

Person

Person

Str ggler

Patient

BeJeficiary

wotkability lost

Perfonal disaster



" -

worry & uncertainty

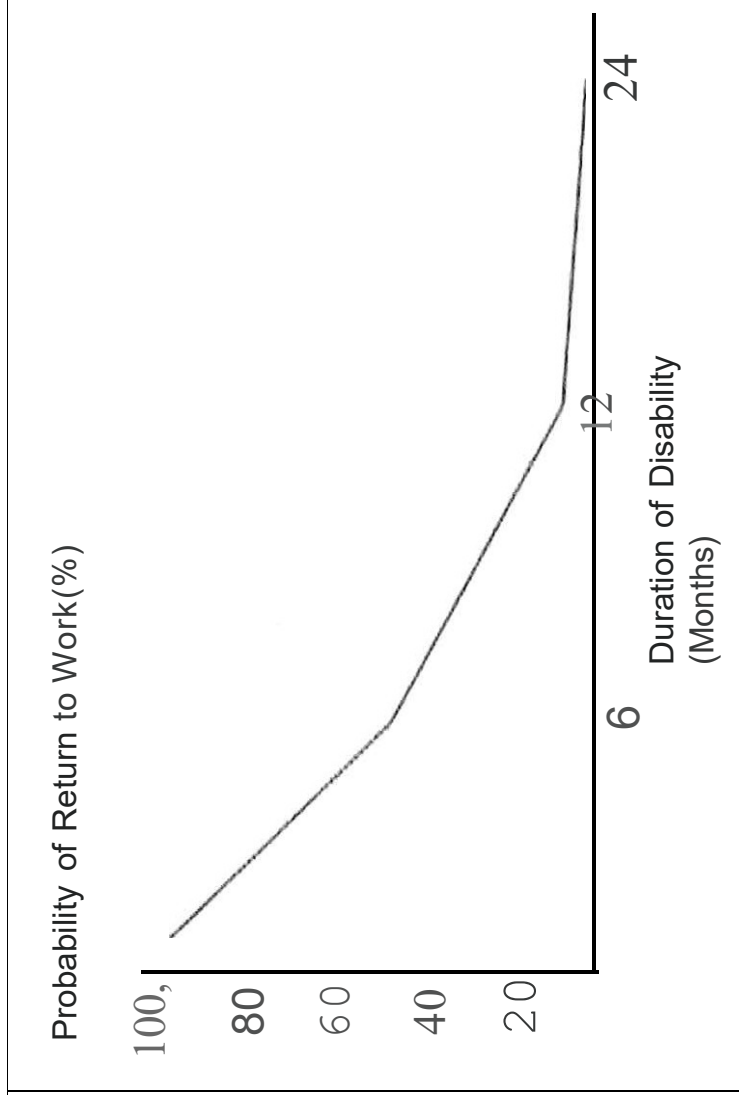
'patient' is a label denoting ownership by clinician - disempowering

similarly disempowering

despondency sets in

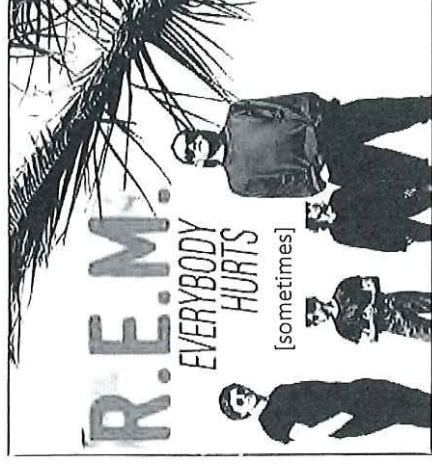
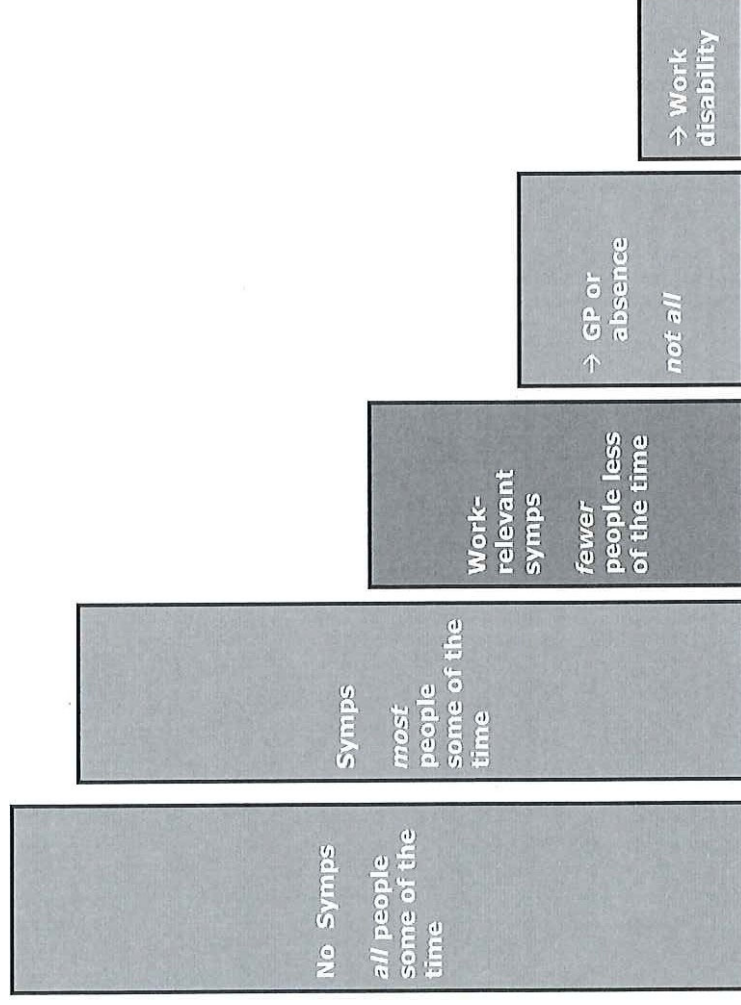
acapted ram hadler

...and it happens very quickly



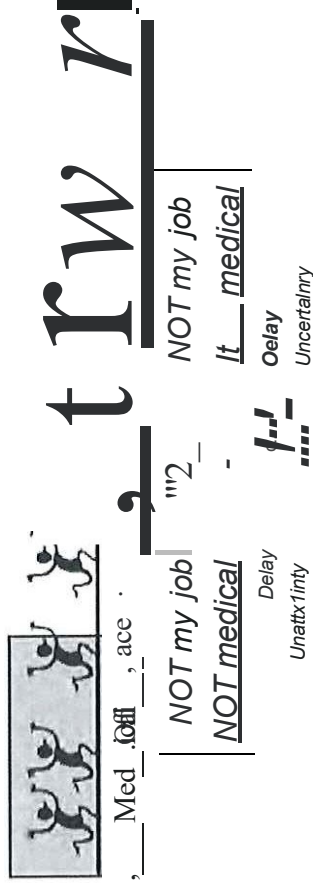
Work=relevant symptoms

- the key to understanding



- People with certain job demands may not notice symptoms at work
 - not work-relevant
- People with another set of job demands may struggle with symptoms
 - **Work-relevant symptoms**
 - Usual work temporarily difficult or impossible
- Work-relevance is about this person, in this job, with these symptoms, at this time
 - Highly individual

The Gap: Whose Responsibility IS it?

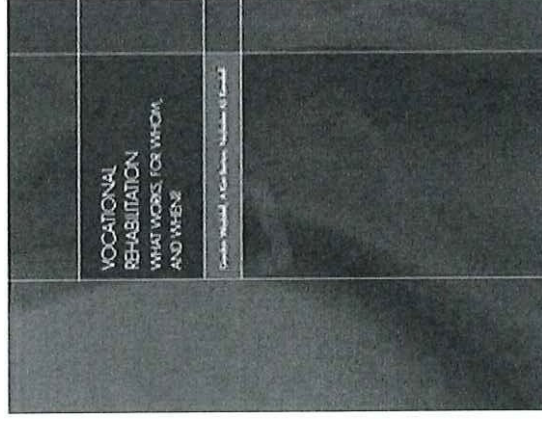


if: ' . f it

**Result: Needless Work Absence., Job Loss.,
Withdrawal from Workforce**

Maybe it's both

- Vocational rehabilitation
 - Work-focused healthcare
 - Accommodating workplace
 - *if* delivered early



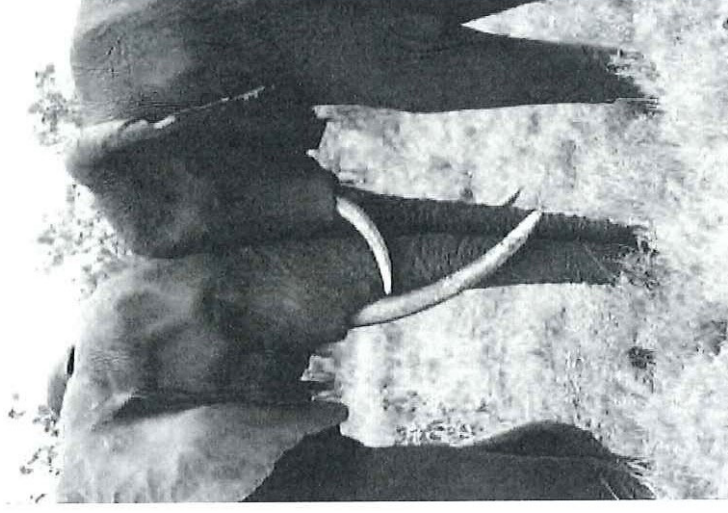
Don't forget the worker

- They know their job
- They know how they feel
- They know what they can and can't do
- Maybe they can safely return sooner than everyone thinks
 - *given the right support*

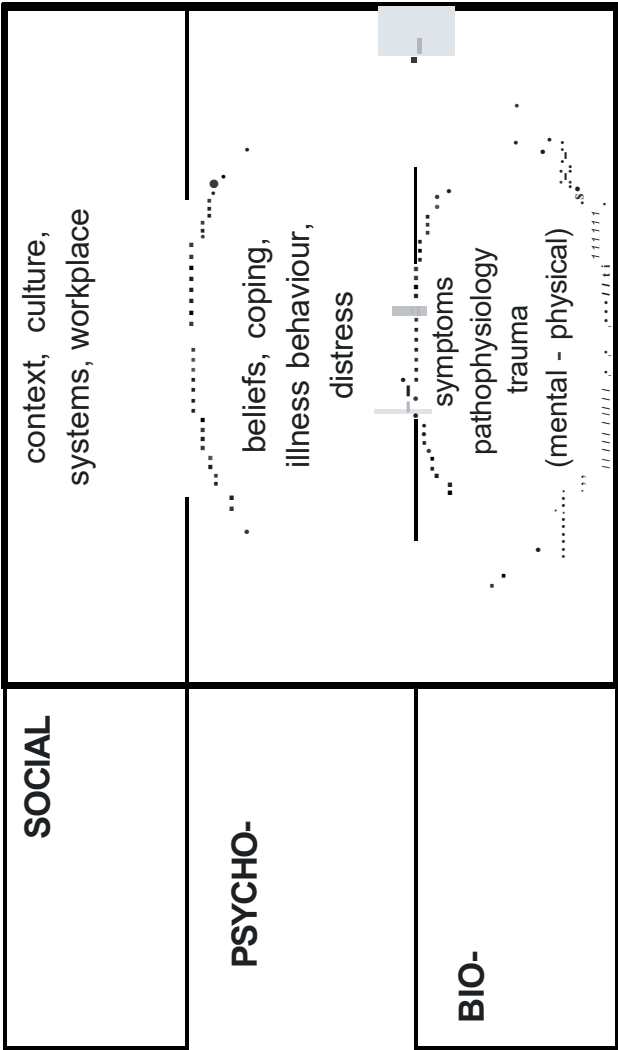


Why do some people slide to disaster?

- Mostly, they do not have a more serious health condition or more severe injury
 - So, it's not about what has happened to them; rather it's about why they don't regain work ability
- They face **obstacles**
 - things that get in the way of returning to normal
- They need help to overcome their obstacles
 - biopsychosocial approach



Biopsychosocial model of disability



The way illness or injury affects us depends on multiple factors

Biopsychosocial model incorporates interactions between person and environment.

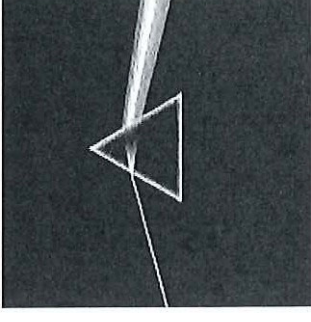
Addresses personal and psychological issues.

Recognises biological factors

Principles and Process

• PRINCIPLES

- Work is an important health outcome
- Physical and mental symptoms common
- Treatment not always needed
- Most people can stay at work
- Some will struggle with work tasks or environment
 - absence appropriate if work intolerable
- Early return to work beneficial
 - avoids disability



• PROCESS

- Accurate consistent information/advice
- Stepped care approaches
 - just what's needed when it's needed
- Work focused healthcare
- Supportive workplace
- High fidelity to evidence-based concepts is **crucial**
 - coordination between healthcare, workplace, worker, and society
- All players onside
 - kicking in the same direction

What's the evidence on return-to work interventions and who delivers them?

- The what
 - Early identification of obstacles to return-to-work
 - Determine solutions to obstacles to facilitate early return to work
 - person-workplace-social solutions
 - temporary job modifications
 - Create a Return-to-Work Action Plan
 - Negotiate the Plan with workplace and healthcare
 - Agree the Plan
 - Implement Plan and monitor
 - The who & how
 - Primary healthcare
 - no health-work conversations
 - Occupational Health
 - only available to 50%
 - Vocational rehabilitation
 - even less availability
 - Public health education
 - limited distribution
 - Work-focused healthcare
 - needs coordinating
 - Supportive workplace
 - needs coordinating
- And, their interventions don't always have that crucial fidelity to the evidence

Policy message: *principles as well as process*

- It's all too easy to deliver a useless intervention very efficiently
- And, as we've found out with Fit Note and Fit for Work service, the opposite can apply- great interventions can be delivered inefficiently!
 - Both ineffective
- cost-beneficial outcomes arise from delivering the right stuff in the right way
- policy makers are catching up
 - but they need the right tools

• If we simply focus on HOW we deliver (process measures), all looks good a false sense of worth

- we need to put more effort into WHAT we deliver
 - high fidelity to the evidence



We need to shift the culture

- the Widget Revolution

- Tackling needless work disability is **a systems** issue
- Insurers can do much to help that shift
 - A lot can be done at (relatively) little cost
 - If we adopt the right conceptual framework and a universal consistent delivery model



The trick is... something quite different

- **An innovative implementation**
tool that allows the right stuff to
be delivered to the right people
at the right time
- **Empower the overlooked player**
 - the worker/claimant



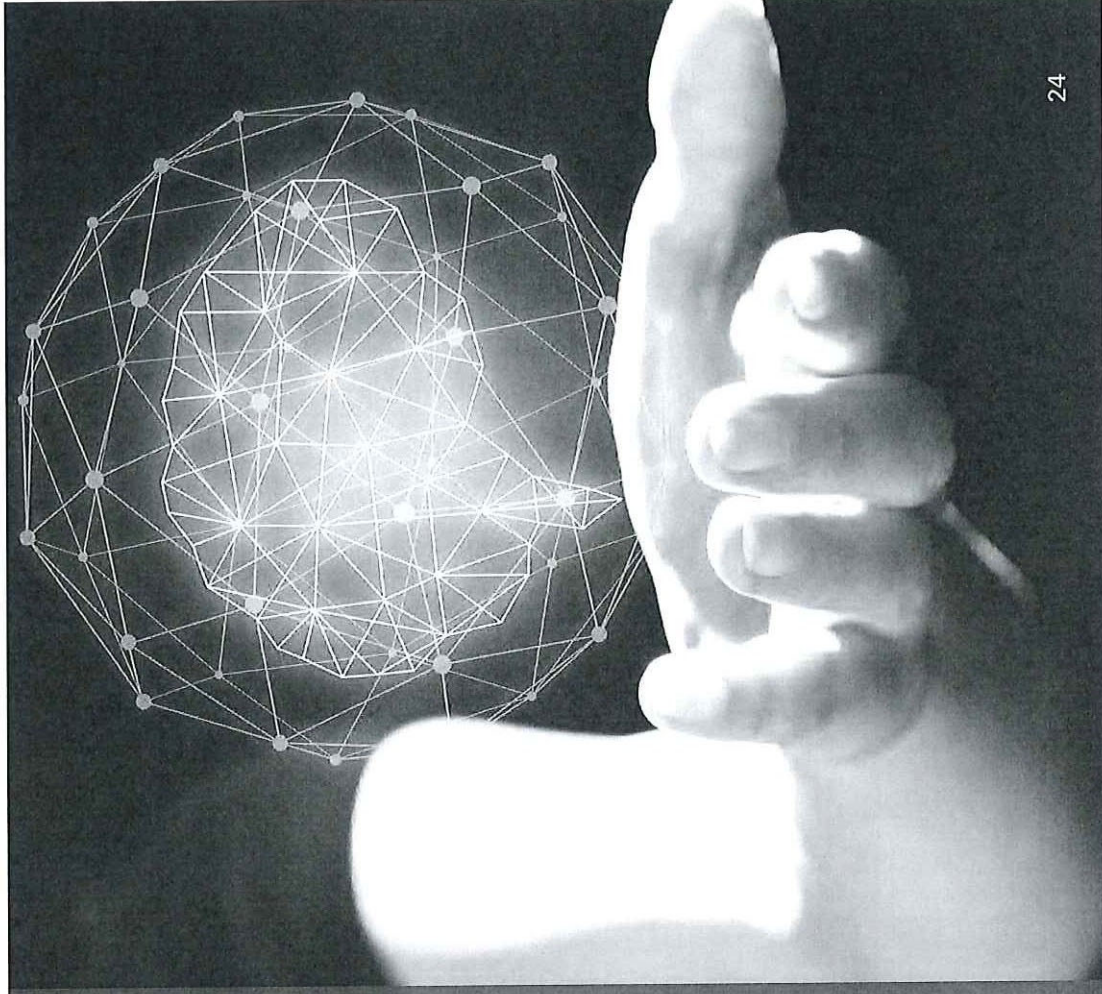
- This is where Widget
comes in



- as Serna will explain....

Dr Abasiama Etuknwa,
University of Huddersfield

The RTW tool



Knowledge Transfer Partnership (KTP) programme

- Implemented using an intermediary - the KTP Associate

University of
HUDDERSFIELD
Inspiring global professionals



We work with
Innovate UK

Aims, solution & key evidence-based messages: *what* works and *why*



Aim: develop and implement an evidence-based individual-focused RTW support tool in the *early stages* of absence.



Our solution: a web app to empower workers on their RTW journey, using knowledge transfer processes, including co production with key stakeholders.



Good work is important for good health



Prolonged absence leads to work disability and poorer health



Early RTW is health supportive

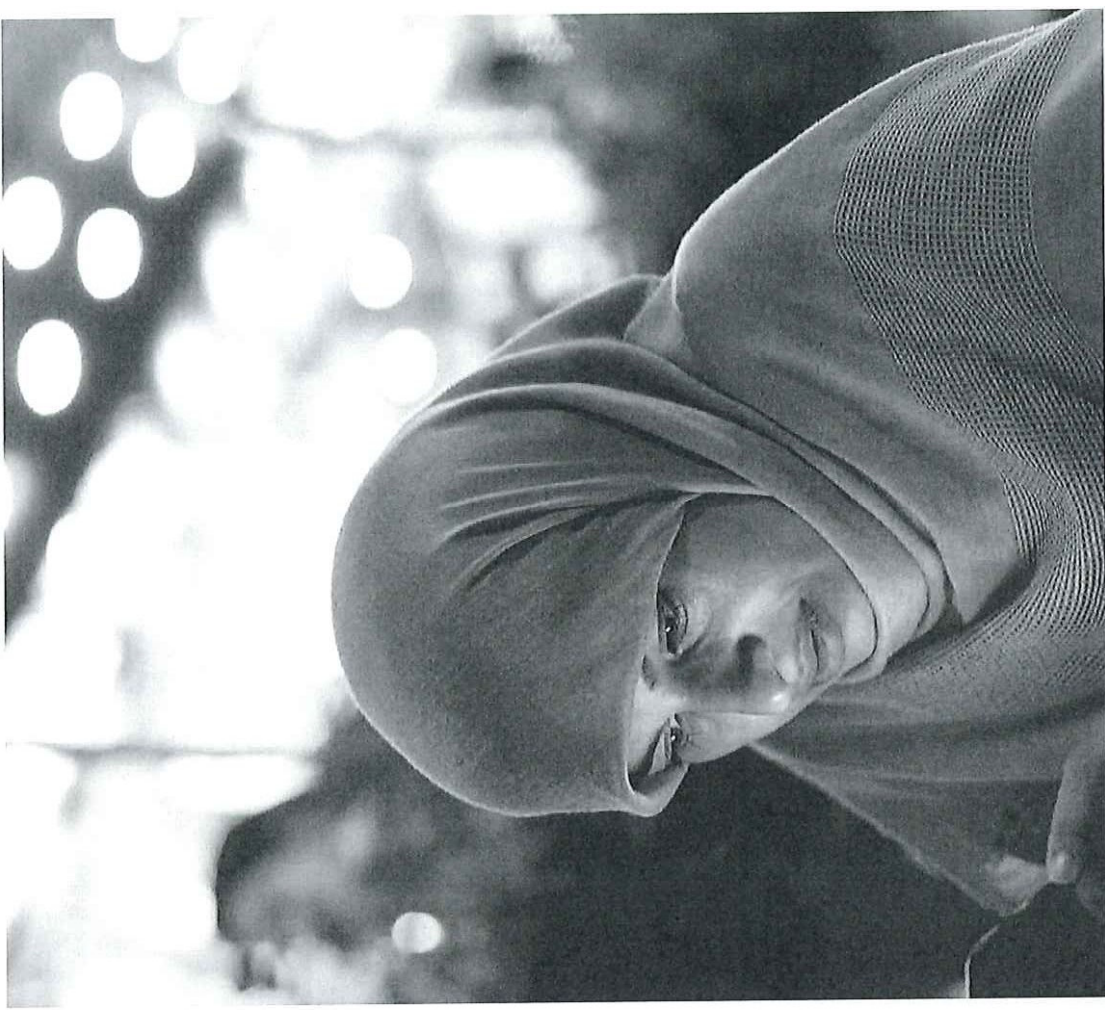
What's needed:

- communication & collaboration
- with the workplace & health professionals
- using a *RTW Action Plan*

Key points

Working whilst recovering

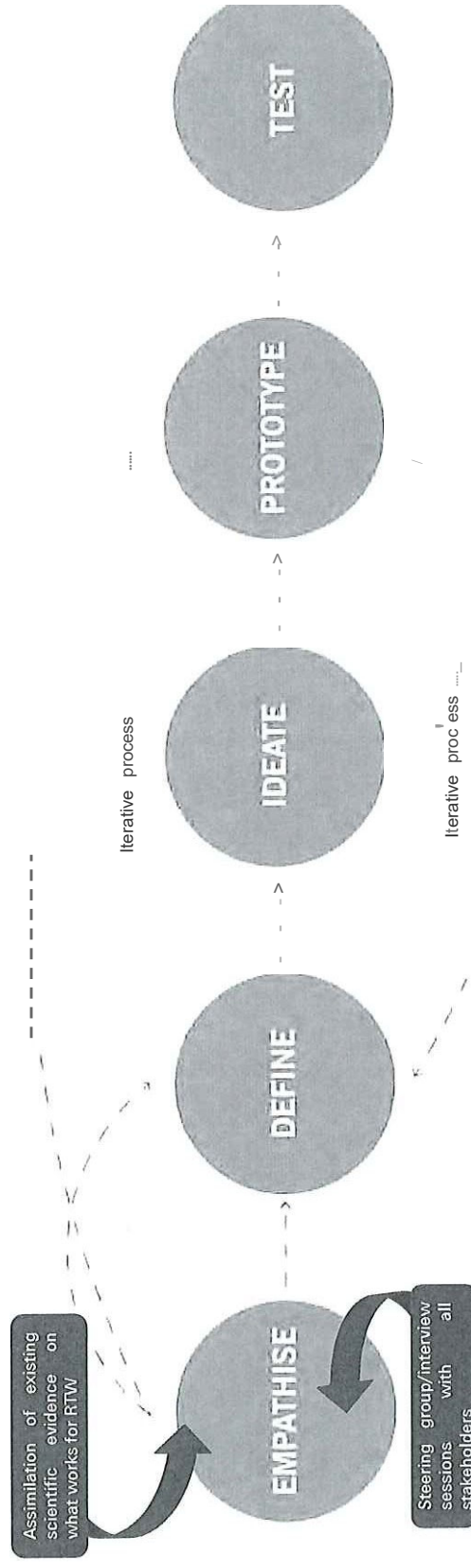
- Tailored to individual
- Within first 4 weeks of absence
- Aim: equip individual to effectively devise and manage their **own** *RTW Action Plan*
- Doesn't replace current RTW practices
- Agnostic of industry, condition or job role
- Equally applicable to group/individual DI
- Online tool
- Novel approach
- Current prototype phase - pilot next phase



Key differences with other digital health & wellbeing tools

Uni of Huddersfield collaboration		Others
Format		<i>Mobile apps</i>
Focus & Content		Educational materials covering broad areas of <i>health & wellbeing</i>
Engagement		<i>Passive</i> engagement
Outcome		<i>Education</i> through learning materials

The co-production process



Stakeholder engagement & feedback

150 stakeholders involved in ideation - incl. gov't policymakers

100 stakeholders involved in prototype testing to date (ongoing)

Claimants

"I'm starting to think about RTW, and this would be a useful way to go about it."

"I would have definitely welcomed a tool like this during my RTW journey."

"A tool like this could have helped prevent the many failed attempts at RTW I had."

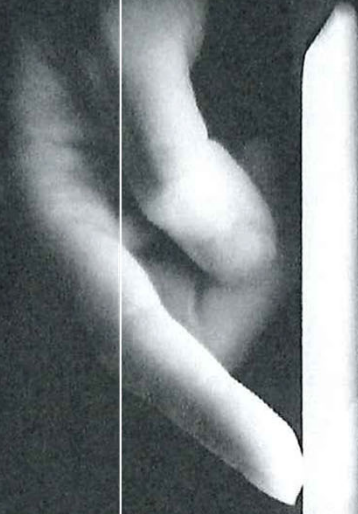
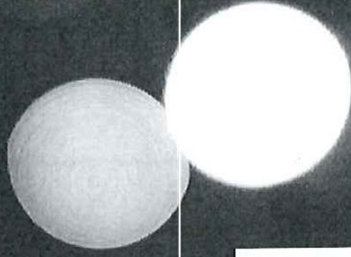
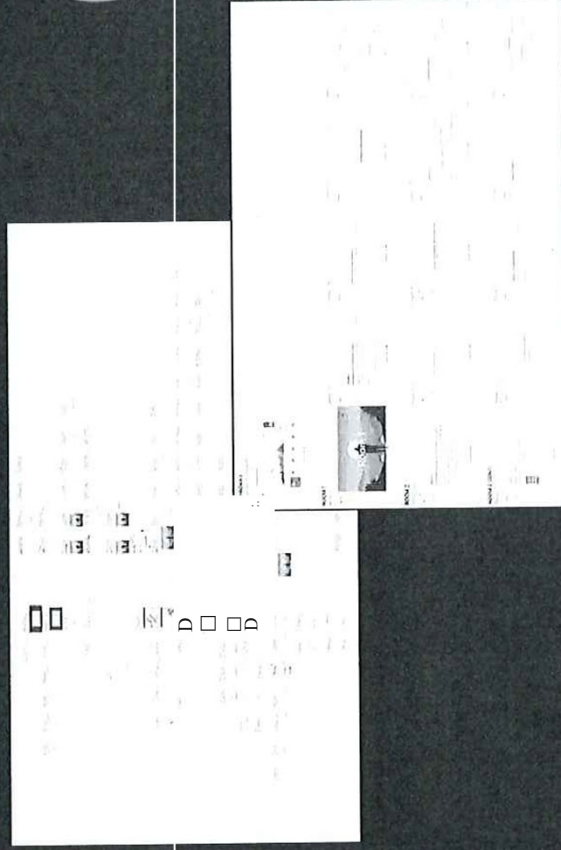
Medical professional

"Can see the benefits of the tool and how it would fit in with managing cases at the clinic."

Academic in work & health field

"The methodological approach adopted here positions this project at the credibility level of being eligible for high profile research funding (such as via NIHR - National Institute for Health & Care Research)".

Prototype demo



Your thoughts

 Swiss Re



Question 1

- Do you think this tool could help - and help empower - a worker on their return to work journey? How/why?

Question 3

- Can you see any challenges involved in using this tool?

Question 2

- How would this tool add to your existing practice?

Question 4

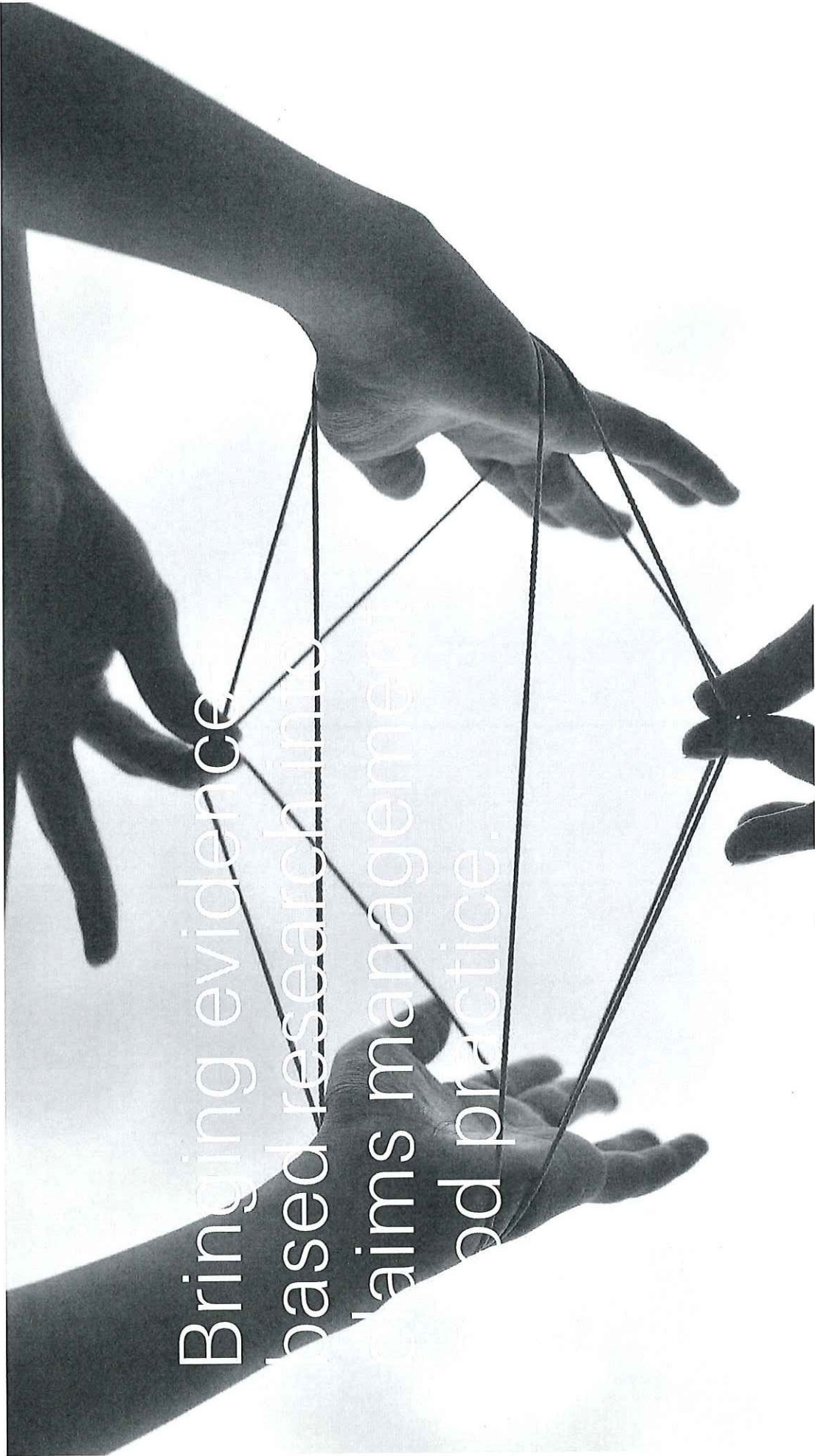
- To what extent do you feel this tool is based on science? (1 to 10 scale)

Question 5

- To what extent do you think this tool could benefit:
 - Insurance practice?
 - Employers?
 - Rehabilitation provider practice?
 - Wider society?(1 to 10 scale for each)

Question 6

- To what extent do you feel you could 'sell'/promote this tool to:
 - Policyholders/claimants (group/individual)?
 - Employers?
 - Brokers/intermediaries?(1 to 10 scale for each)



Bringing evidence-based research into claims management good practice.



Thank you!

Contact us

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Health Claims Forum

ABI Critical Illness Working Group Minimum Standards – Update

Monday 11 July 2022

Presenter

Paul Reddick, Principal Underwriter



PACIFIC LIFE

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- Full review is undertaken every 3 years
- Working group discusses all model wordings and agrees on those conditions that require a review

The main factors driving a review include:

- Changes to medical science/ practices (e.g. are definitions using up to date terminology and reflecting current medical protocols?)
- Addressing issues/ problems experienced by claim assessors (e.g. uncertainty of claim validity based on definition wording, inconsistent decision making)
- Ensuring the definitions provide consumers with a reasonable expectation of a successful claim (e.g. making sure thresholds align with available medical evidence/ protocols and that scope of cover and definitions are clear and unambiguous)
- Ensuring CI products remain sustainable (e.g. reviewing 'severity thresholds', claims can be supported by 'objective evidence', evolving diagnostic/ screening techniques giving rise to increasing trend of low severity claims)

Identified 3 areas for review

G --- ci Dementia / Alzheimer's disease

9 --- o Cancer

@ --- , Heart Attack



Dementia / Alzheimer's disease - new definition

01)

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Dementia including Alzheimer's disease [before age x] resulting in permanent symptoms of specified severity

A definite diagnosis of Dementia, including Alzheimer's disease [before age x], by a Consultant Geriatrician, Neurologist, Neuropsychologist or Psychiatrist supported by evidence including neuropsychometric testing.

There must be permanent clinical loss of the ability cognitive dysfunction with progressive deterioration in the ability to do all of the following

- remember
- reason and
- perceive, understand, express and give effect to ideas

For the above definition, the following -ara is not covered

- Other types of dementia
- Mild cognitive Impairment (MCI)

ABI received feedback from various dementia groups that the definition should not be specific to Alzheimer's disease and should be broadened to all types of dementia - the working group agreed that we should make this change, but to retain Alzheimer's Disease in the body of the definition to help with customer awareness.

The Working group were aware that the wider market already covers all forms of dementia, and acknowledges this change will have minimal impact on the scope of dementia cover available to consumers.

We decided to retain the **types of medical specialists required to make a diagnosis** as it is consistent with the previous definition and acknowledges diagnoses being made in a secondary care specialist environment.

We replaced "**permanent clinical loss of ability**" with "**permanent cognitive dysfunction**" as this aligns with relevant terminology, is easier for customers to understand and retains a specific severity level of permanency.

Neuropsychometric testing has been added to provide an objective assessment of function, and because this is a diagnostic requirement within NICE guidelines.

Mild cognitive impairment is now specifically excluded to underpin the severity threshold and recognise this is a formal diagnosis made for early stage dementia.



Cancer - new definition

@/i

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Cancer -excluding less advanced cases

Any malignant tumour positively diagnosed with histological confirmation and characterised by the uncontrolled growth of malignant cells and invasion of tissue.

The term malignant tumour includes leukaemia, sarcoma, and lymphoma except cutaneous lymphoma (lymphoma confined to the skin) those that arise from and are confined to the skin (including cutaneous lymphomas and sarcomas).

For the above definition, the following are not covered:

- All cancers which are histologically classified as any of the following:

- pre-malignant;
- non mvas,ve
- cancer in situ;
- having borderline malignancy; or
- having low malignant potential;
- All tumours of the prostate unless histologically classified as having a Gleason score of 7 or above or having progressed to at least TNM classification cT2bN0M0 or pT2N0M0 following prostatectomy (removal of the prostate).
- Chronic lymphocytic leukaemia unless histologically classified as having progressed to at least Binet Stage
- Allurothelial tumours unless histologically classified as having progressed to at least TNM classification T1N0M0.
- Gastrointestinal stromal tumours and neuroendocrine tumours without lymph node involvement or distant metastases unless they are WHO Grade 2 or above.
- Any skin cancer (including cutaneous lymphoma) All cancers that arise from and are confined to one or more of the epidermal, dermal, and subcutaneous tissue layers of the skin (including cutaneous lymphomas and sarcomas), other than malignant melanomas that has been histologically classified as having that have caused invasion beyond beneath the epidermis.
- All thyroid tumours unless histologically classified as having progressed to at least TNM classification T2N0M0.



Cancer - reasons for changes

@Li)

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Overview

The Working group acknowledges that the cancer definition is unavoidably complex. The current changes are primarily aimed at providing more clarity for consumers as to what is and is not covered rather than materially changing the scope of cover.

Regular market feedback is that there is difficulty and a lack of consistency in assessment of certain cancer claims, most notably some skin cancer variants, NETs, GIST, and some urothelial tumours; specifically, problems with determining 'invasion' and 'malignant behaviour', even with CMO input (that also varies as to outcomes). Claim assessors are using assessment criteria (e.g. mitotic rate, Ki67 expression) that aren't within the scope of the definition wording to determine claim validity.

Chronic Lymphocytic Leukaemia (CLL)

Removal of the exclusion relating to **Chronic Lymphocytic Leukaemia - Binet Stage A**. The Binet stage is typically used in the UK and the minimum diagnostic stage is A. The Rai index uses stage 0-4 and is typically used in the USA. Binet stage A is broadly the equivalent of Rai stage 1. The original purpose of setting a minimum diagnostic level of Binet stage A was to avoid claims for very early stage disease classified as Rai stage 0. This is now deemed to be superfluous.

GIST/ NET

A minimum severity criteria of WHO Grade 2 is introduced. This reflects that Grade 1 tumours have an excellent prognosis (even with higher T stages in most cases). This change will create a clear baseline for claim validity for assessors and improve clarity of cover for consumers. A review of the market showed that low grade tumours are the most troublesome for claim assessors and are prone to inconsistent outcomes for consumers. Any GIST/ **NET** with lymph node or metastatic spread is covered, even if Grade 1.

Merkel Cell tumours were not included, as the new specific exclusion to NETs would apply and also the amended skin cancer definition.



Cancer - reasons for changes

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Prostate

The TNM staging system was updated in 2017/2018 (shortly after the previous ABI CI guide was issued). This update removed pathological stage pT2a-c and replaced it solely with pT2. This (pathological) stage is assigned following (radical) prostatectomy. We can no longer use the previous minimum stage of T2b for pathological stage. The working group debated several options and decided to adopt a split approach that retains cT2b and uses pT2. It is acknowledged this lowers the threshold for pathological staging compared to the previous version.

The group also discussed alternative staging systems that are occasionally seen at claim such as UICC and is aware that additional claims may be paid using such systems that would not be valid under AJCC TNM staging system.

Skin

Feedback supports that there are difficulties in assessing claims for some non-melanoma skin cancers. The previous wording was repetitive and unclear as to the intended scope of covered skin cancers. Conditions such as DFSP regularly caused difficulties for claim assessors. Better clarity was required in respect of sarcomas. The intention of the definition is that the only covered skin cancers are melanoma of defined severity and non-melanoma cancers that have spread beyond the epidermis, dermis and subcutaneous tissue layers. The new wording addresses the various issues noted by claim assessors and will provide greater clarity for consumers as to the scope of cover in this area.

Urothelial

Feedback supports there are occasional disputes for claims for **papillary** urothelial tumours (mostly urinary bladder) staged pTa. Whilst these are classified as 'non-invasive papillary urothelial carcinoma', specialists often argue that these are 'invasive within the epithelial lining'. Changing the minimum stage to pT1 removes this ambiguity and provides improved clarity for claim assessors and consumers, whilst aligning the minimum severity level with other covered cancers.



Heart Attack - new definition

@JH

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Heart attack - of specified severity

Death of heart muscle due to inadequate blood supply A definite diagnosis of acute myocardial infarction with death of heart tissue as evidenced by all of the following:

- Typical clinical symptoms (for example, characteristic chest pain).
- New characteristic electrocardiographic changes or new diagnostic imaging changes
- The characteristic rise of cardiac enzymes or Troponins recorded at the following levels or higher:
- Troponin T > 200 ng/L (0.2 ng/ml or 0.2 ug/L)
- Troponin I > 500 ng/L (0.5 ng/ml or 0.5 ug/L)

The evidence must show a definite acute myocardial infarction.

For the above definition, the following are not covered:

- Other acute coronary syndromes.
- Angina without myocardial infarction
- Myocardial injury



Heart Attack - reasons for changes

01)

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Background:

The revised wording broadly seeks to align with the 4th Universal Definition of AMI 2018 by use of content and language, whilst recognising the difference between the Universal Definition purpose being to define appropriate clinical pathways and improve patient prognosis and the CI definition wording that intends to pay claims for a definite diagnosis of AMI of specified severity.

The previous wording only covered AMI where there was 'inadequate blood supply'. The group recognise that AMI can occur in the setting of adequate blood supply without sufficient oxygen levels (type II AMI). The new wording incorporates cover for both situations where a definite diagnosis of AMI is made.

The Working Group recognised the importance of cardiac imaging in the diagnostic pathway for AMI, and have now incorporated this into the definition. Imaging is a useful tool in determining cell death in a pattern consistent with AMI. The group is aware that claim assessors currently use imaging as a determinant when there is very minor troponin elevation and/ or equivocal ECG findings to enable payment of claims that might otherwise not be paid.

The new definition **retains the same troponin levels** for consistency with the previous definition and the importance in triaging, and also **chest pain** as customers relate to this as a common indicator for AMI

It now includes **myocardial injury** as a specific exclusion, which has replaced 'Angina without myocardial infarction'. 'Other acute coronary syndromes' remains excluded (in this context it refers to unstable angina' as STEM! and NSTEMI are included as AMI forms of ACS).

Summary

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All discussions within the CI Working Group were compliant with competition law and avoided discussions relating to pricing impact, impact on claims volumes, changes to proportions of claims and individual insurer/reinsurer philosophies and products.

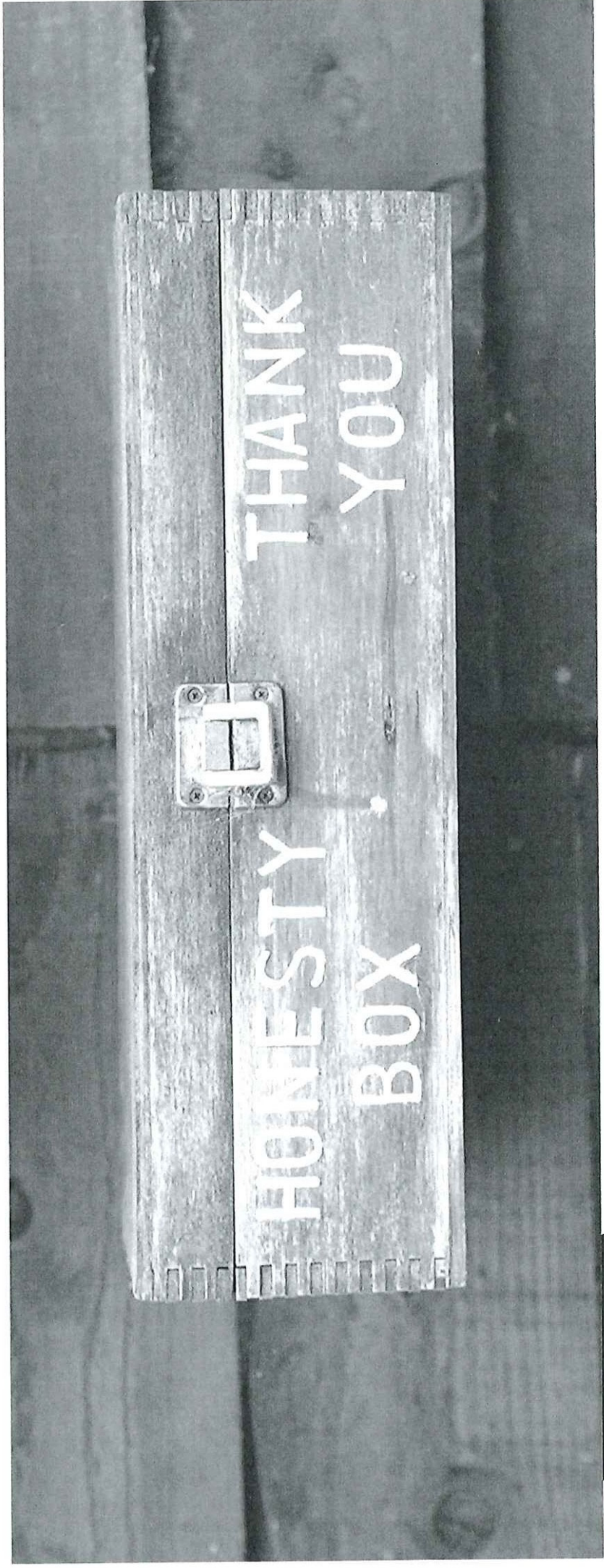
The ABI were made aware of this presentation for the Health Claims Forum, and approved this prior to today's event.

The Working Group has proposed the changes detailed in the previous slides, to provide improve clarity for claim assessors and consumers in specific areas where feedback has indicated uncertainty of the scope of cover and associated claim outcome, with resulting difficulties for both claim assessors and consumers.

The changes will allow greater consistency and certainty of outcomes. The Working Group acknowledges that in respect of the cancer definition changes, there is a perception that this is being further tightened to reduce claim payments. This is not the intention, and the proposed changes are being made to reflect the current market position on claim outcomes in situations where the majority of claims are usually declined for not meeting the definition due to the ambiguity in the previous wording. The Working Group recognises that Insurers remain free to enhance the level of cover beyond the ABI Minimum Standards as set out in the definition(s).

Thank you for listening

I am pleased to take any questions



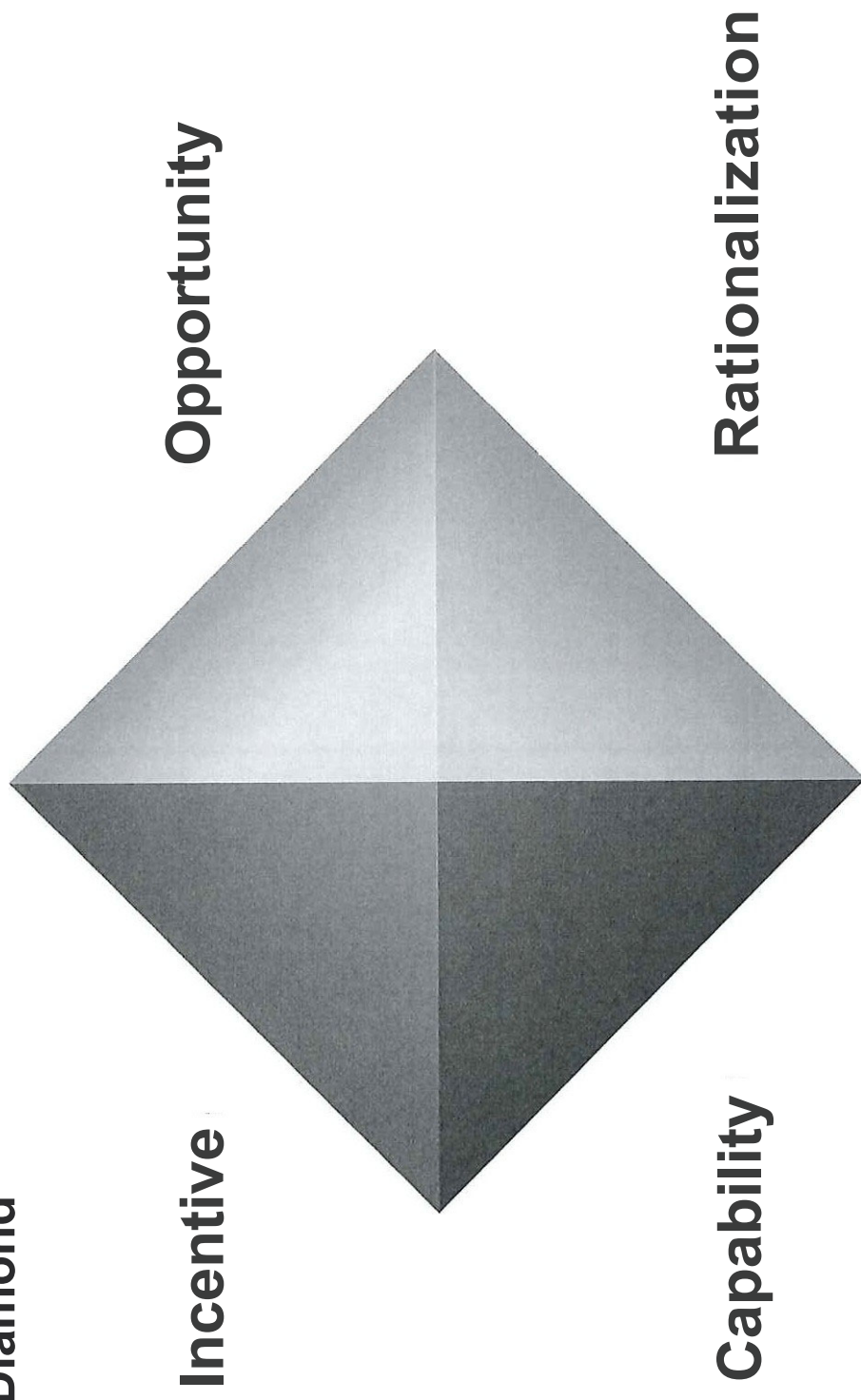
The psychology of fraud

RGA

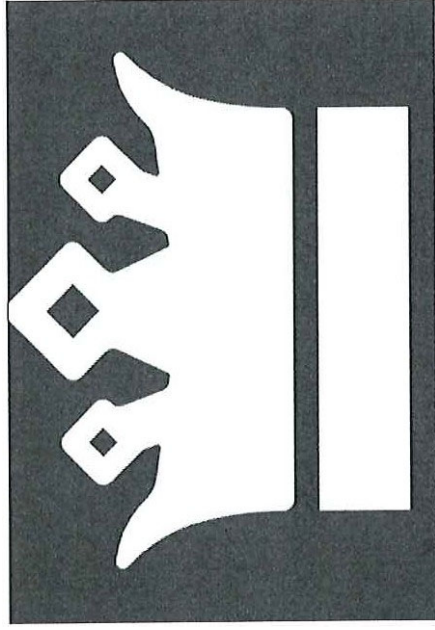
Irrationally honest



The Fraud Diamond

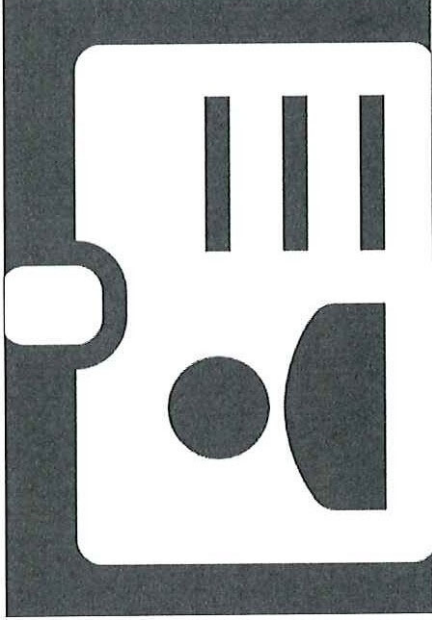


Rationalization



ENDOWMENT

We value a good more highly if it is ours or we feel entitled to it



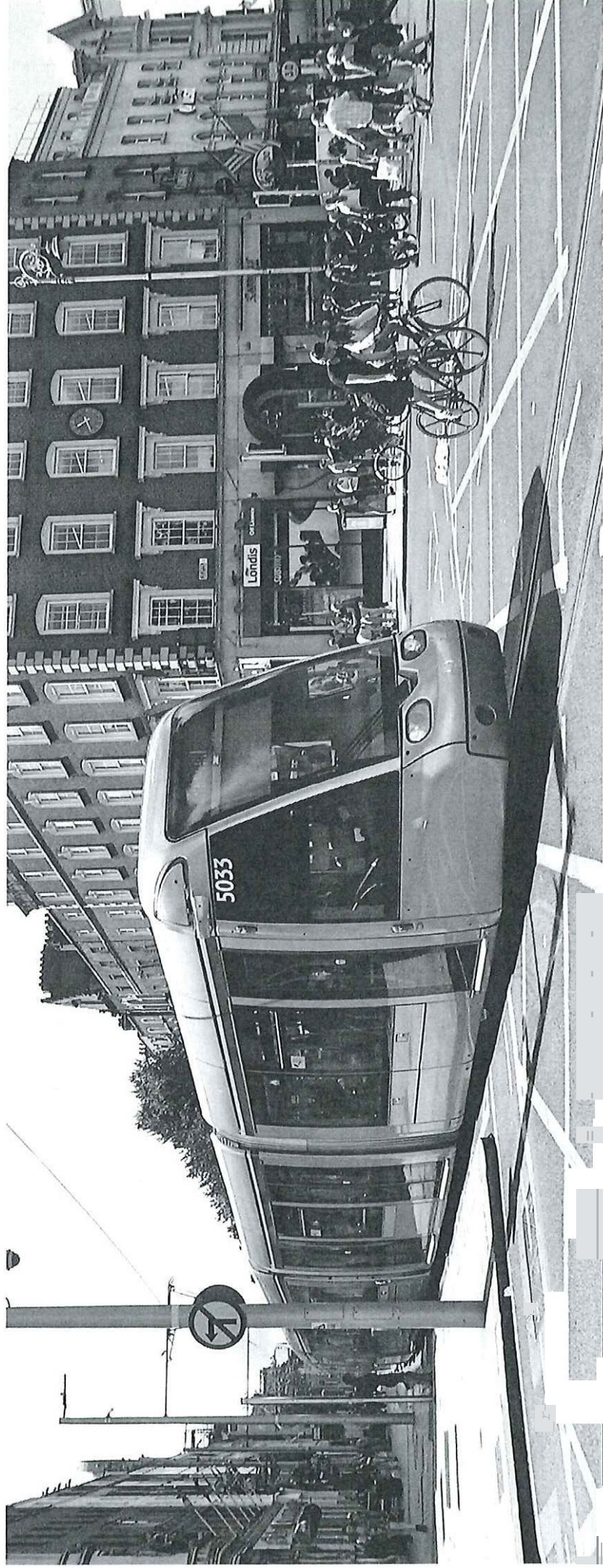
IDENTITY

We are motivated to maintain a self-image consistent with characteristics, roles and affiliations we hold dear



NORMS

We are influenced by behaviours of multiple comparable others



Tackling fare evasion on public transport

RG&A

Tackling fare evasion on public transport



Tackling fare evasion on public transport



Tackling fare evasion on public transport



TICKETS PLEASE

Inspections
are being made by
Revenue Officers
in uniform and
plain clothes.

LUAS

Transport
for Ireland

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Over 95% of
Luas passengers
buy a ticket

When asked what word you'd use to
describe people who don't, the most
common response was...

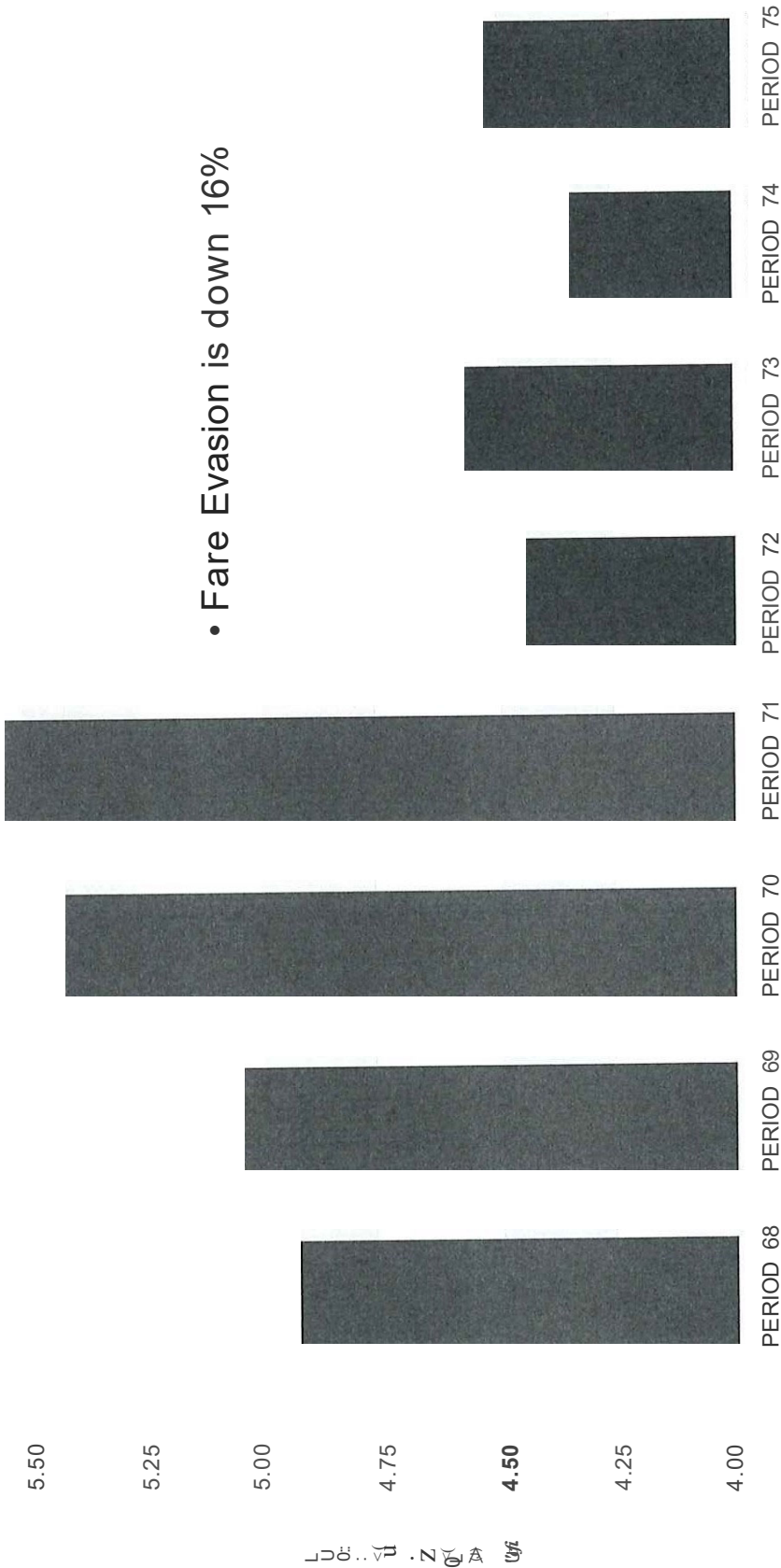
j **Cheater***

*Submitted on Luas by 147m, J. U. sry 201e.
Please guard...ntly verified by third p...s.

LUP \S

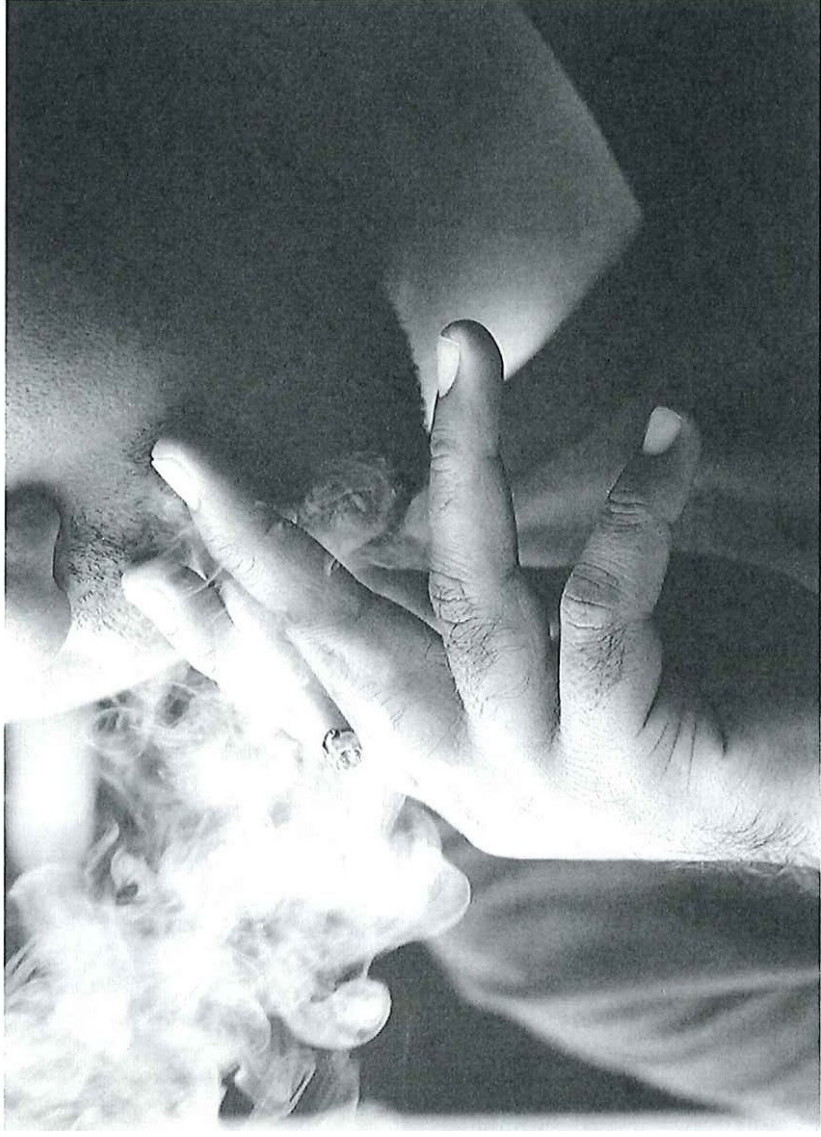
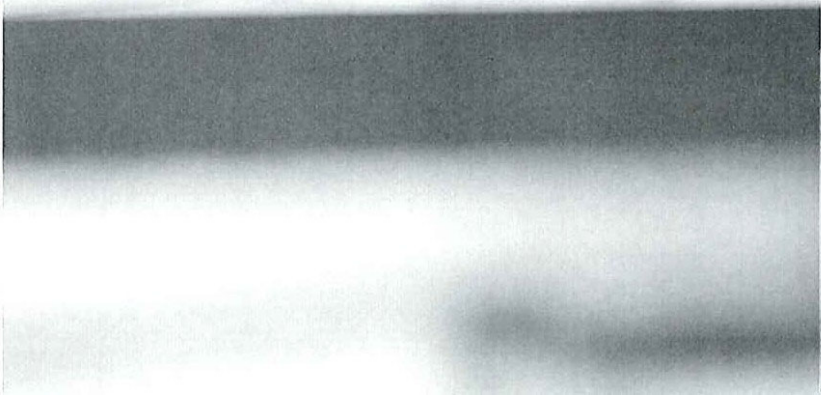
 ITI
INTEGRITY
TRANSPARENCY

Tackling fare evasion on public transport



New South Wales, Australia





Increasing the honesty and accuracy of
underwriting disclosures

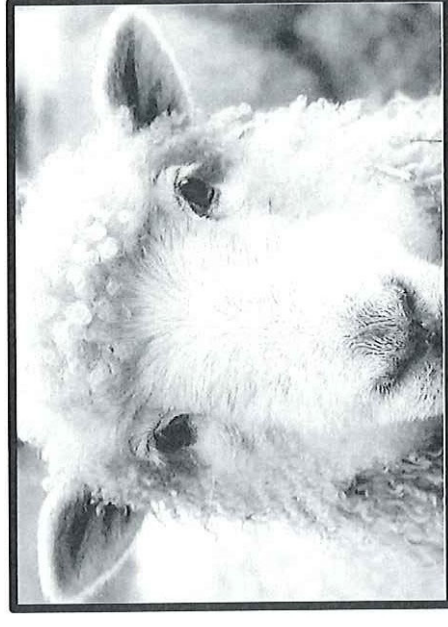
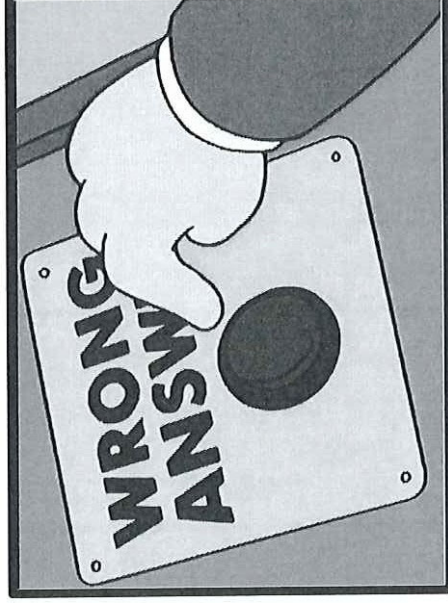
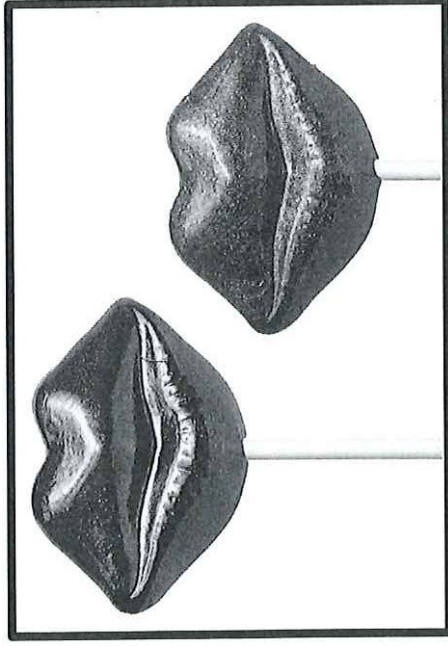
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Understanding the disclosure gap

- Intentional inaccuracy
 - Lying
 - Economic benefit - ensuring coverage and reducing premiums
 - Psychological benefit - avoiding embarrassment or distress
- Unintentional inaccuracy
 - Not knowing
 - Forgotten
 - Mistaken



Make it easy to be accurate and hard to lie



Keep it simple

- Reduce cognitive load: language, calls to action, pre-empt questions, chunking
- One question at a time
- Simpler 2: fewer

Don't make it obvious

- Don't make it easy for those who want to cheat
- Underwriting rule "# question
- E.g. Did you smoke in the last 12 months?

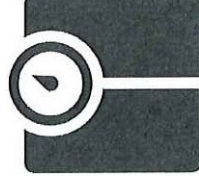
Beware the implied norm

- Questions are rarely neutral
- Avoid stigma, embarrassment, guilt and shame
- Normalize normal behavior

Key questions



Drugs

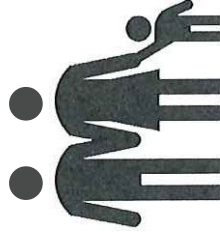


Height &
weight

ft a



Alcohol



Family
history



Smoking



Medical
conditions

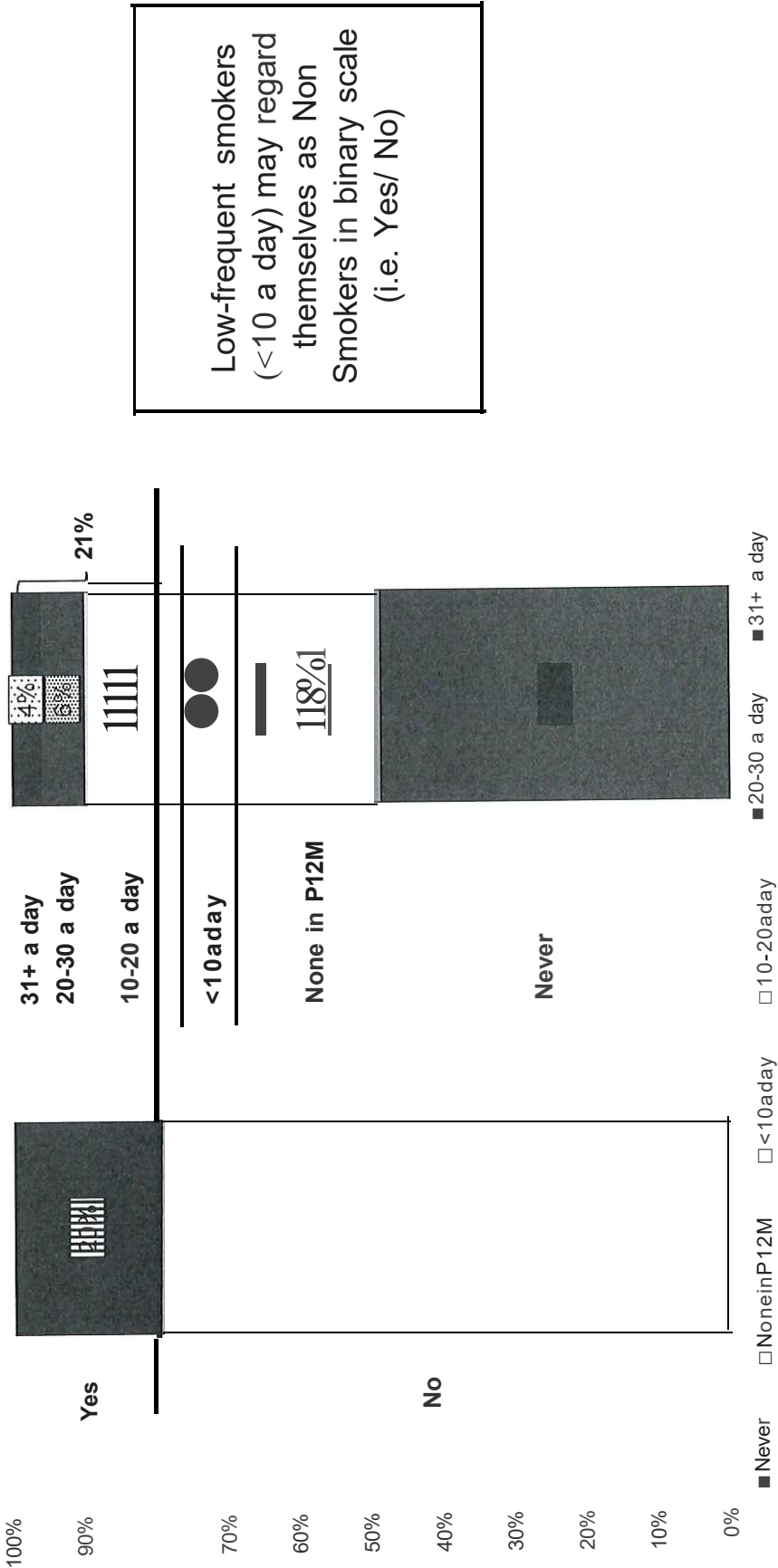
Smoking: typical question

Have you ever used tobacco
or nicotine substitute in any
form?

Yes

~~No~~

Smoking: Change identity and implied norms



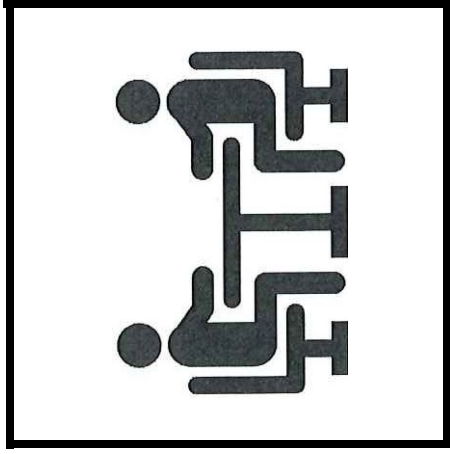
Smoking: Behaviorally enhanced

When did you last use a tobacco or other nicotine products in any form?

- In the past month
- In the past year
- Between 1-5 years ago
- More than 5 years ago
- Never

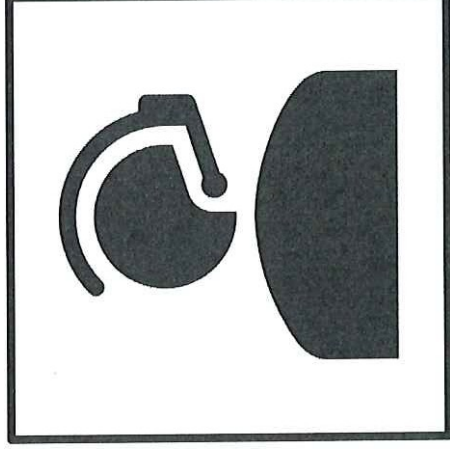
- Avoid binary questions which make the underwriting rule obvious.
- Destigmatize by assuming the behavior. Ask 'when did you?' rather than 'have you?'.
- Provide multiple options, weighted towards the higher frequency and recency rates.
- Can reduce the total number of questions.

Understanding the Disclosure Gap



Adviser led

- Explanation but not always understanding
- Incentives and embarrassment



Tele

- Explanation and understanding
- Embarrassment



Online

- Convenience and time
- Explanation but not always understanding
- Impersonal but still some embarrassment

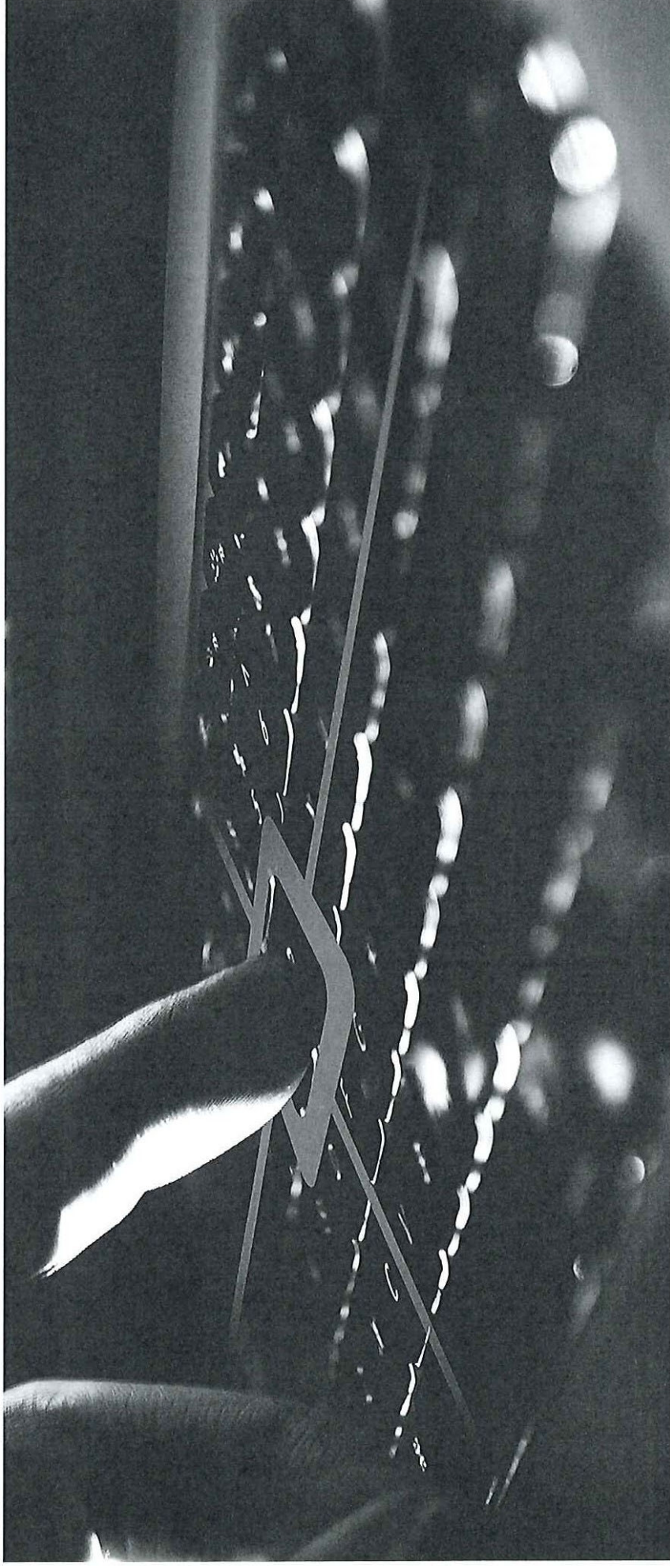
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Reducing claims fraud	
RGA	

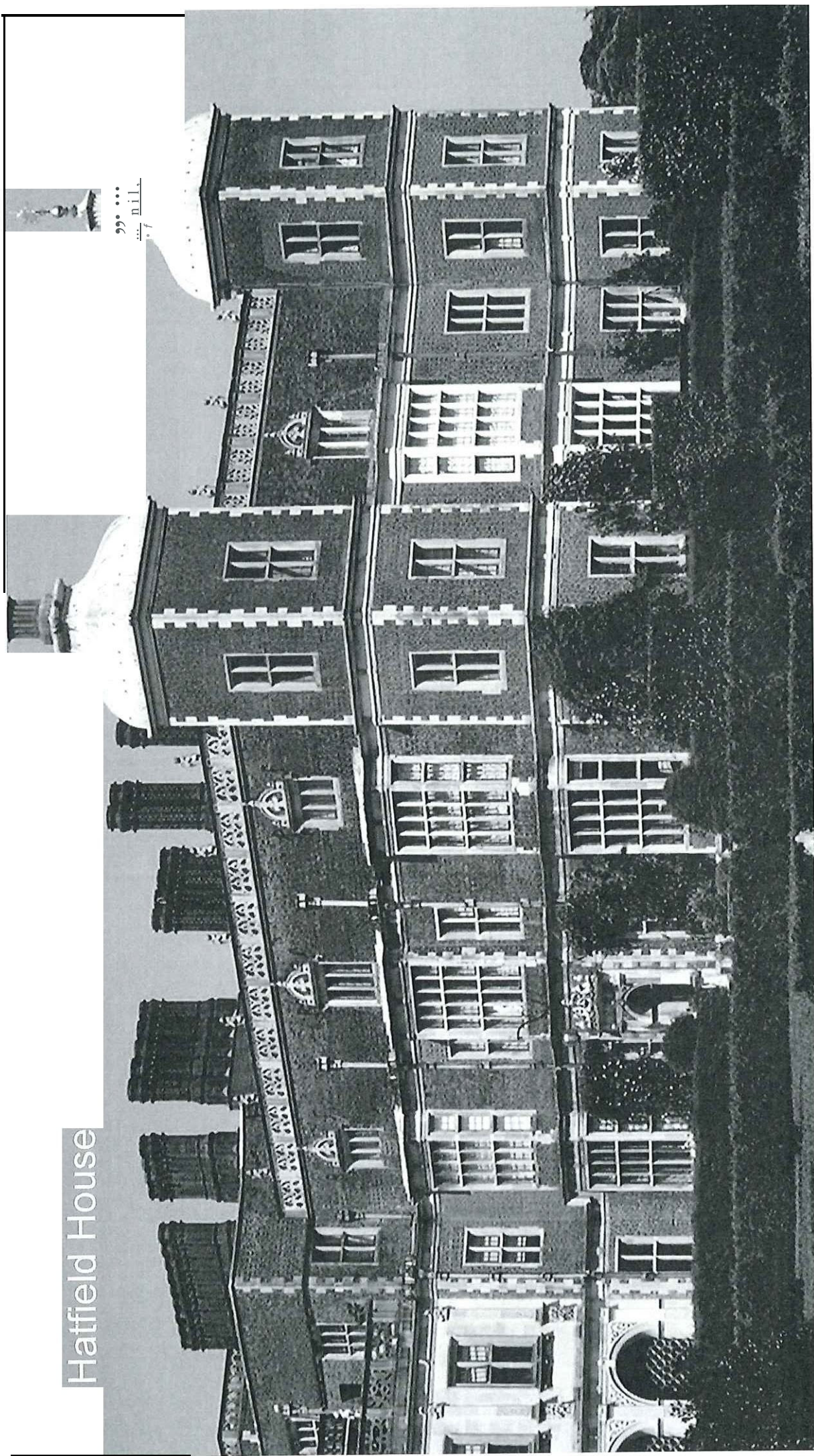
Name and shame



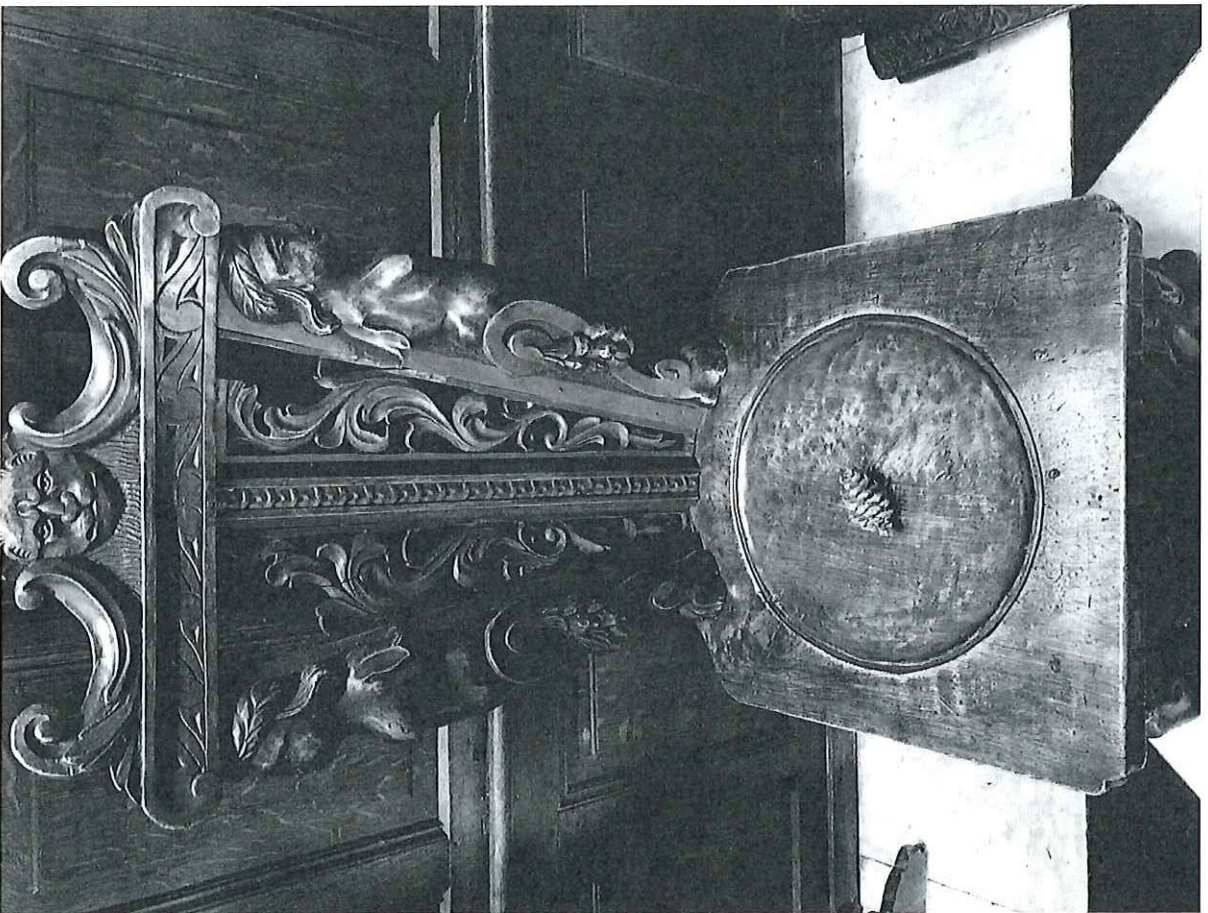
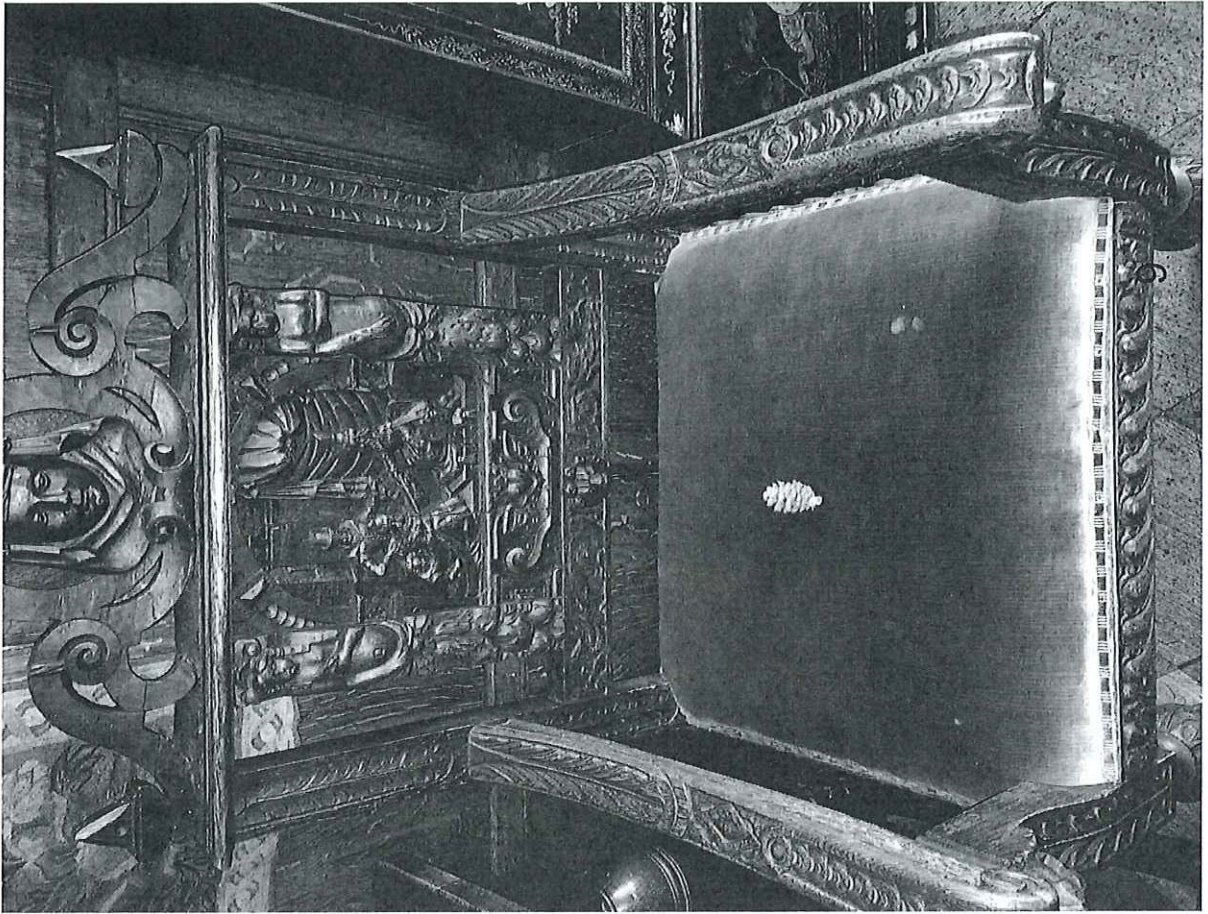
Behavioural analytics



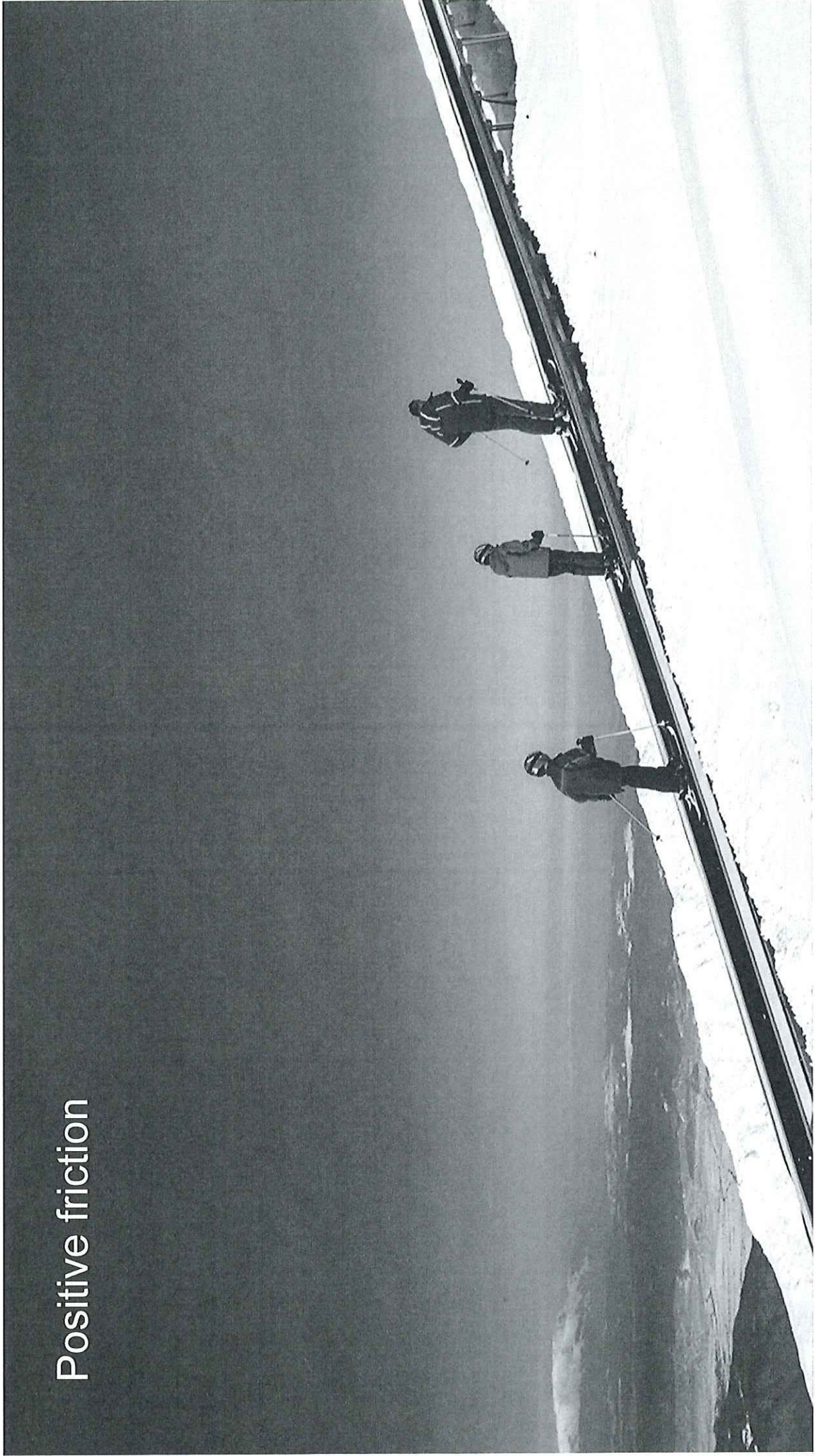
Hatfield House

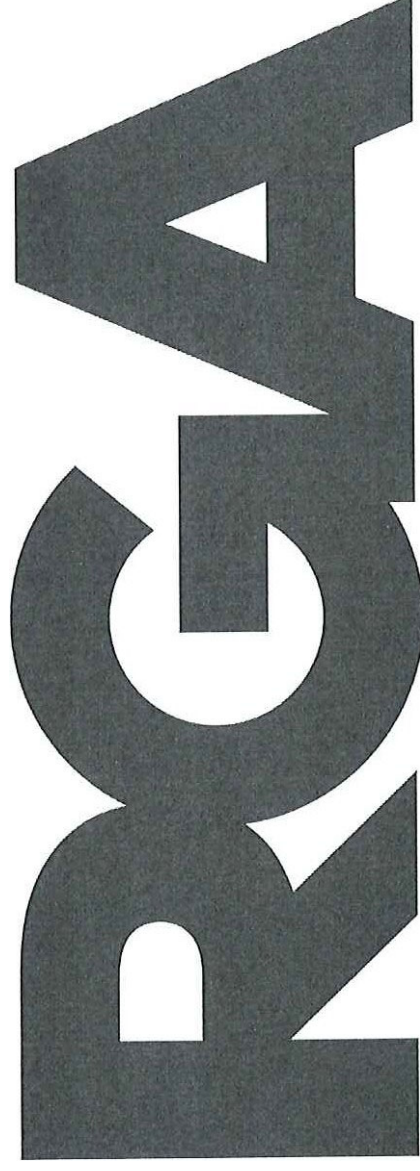


99...
... nil.
7



Positive friction



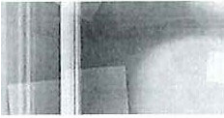


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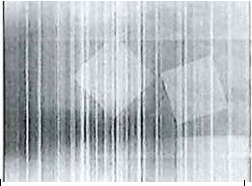
Why we lie



The psychology of fraud
and impact of COVID

Matt Battersby, Chief Behavioural Scientist

11th July 2022



Scale of Covid loan fraud leaves UK struggling to reclaim billions lost

While prosecutors focus on organised criminals, many of the vastly inflated their revenue



FT montage: Decamating

Insurance Times

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NEW S

'One insurance scam per minute' during pandemic, warns IFB

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Insurance fraudsters are using the pandemic to make money from unsuspecting consumers

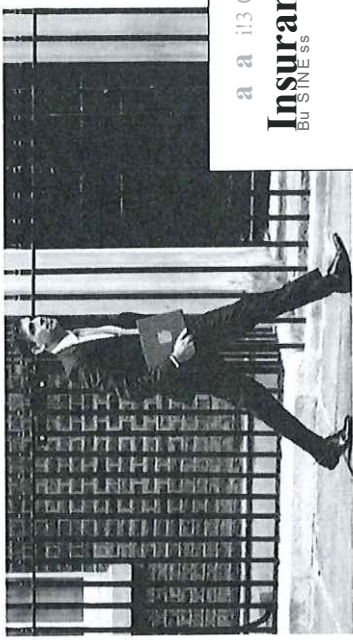
The Insurance Fraud Bureau (IFB) is warning the public to be on the lookout for more than 150 new types of insurance fraud during the Covid-19 pandemic, including face mask fraud, more than 150 new types of insurance fraud.

MAXIMUM SERVICE MINIMUM FUSS

At the heart of insurance

Money lost to Covid fraud and error is more than the 10 biggest heists in history put together

Labour has accused Rishi Sunak of a 'great Covid robbery' because £11.5bn has gone missing from the Exchequer



Rishi Sunak is accused of being careless with public money. (Photo: Leon Neal/Getty)

By Hugo Gye
Political Editor

May 26, 2022 9:00 am (Updated 10:15 am)



Insurance

New Aviva report reveals spike in insurance claims fraud



Why do we behave in the ways that we do?

RGA

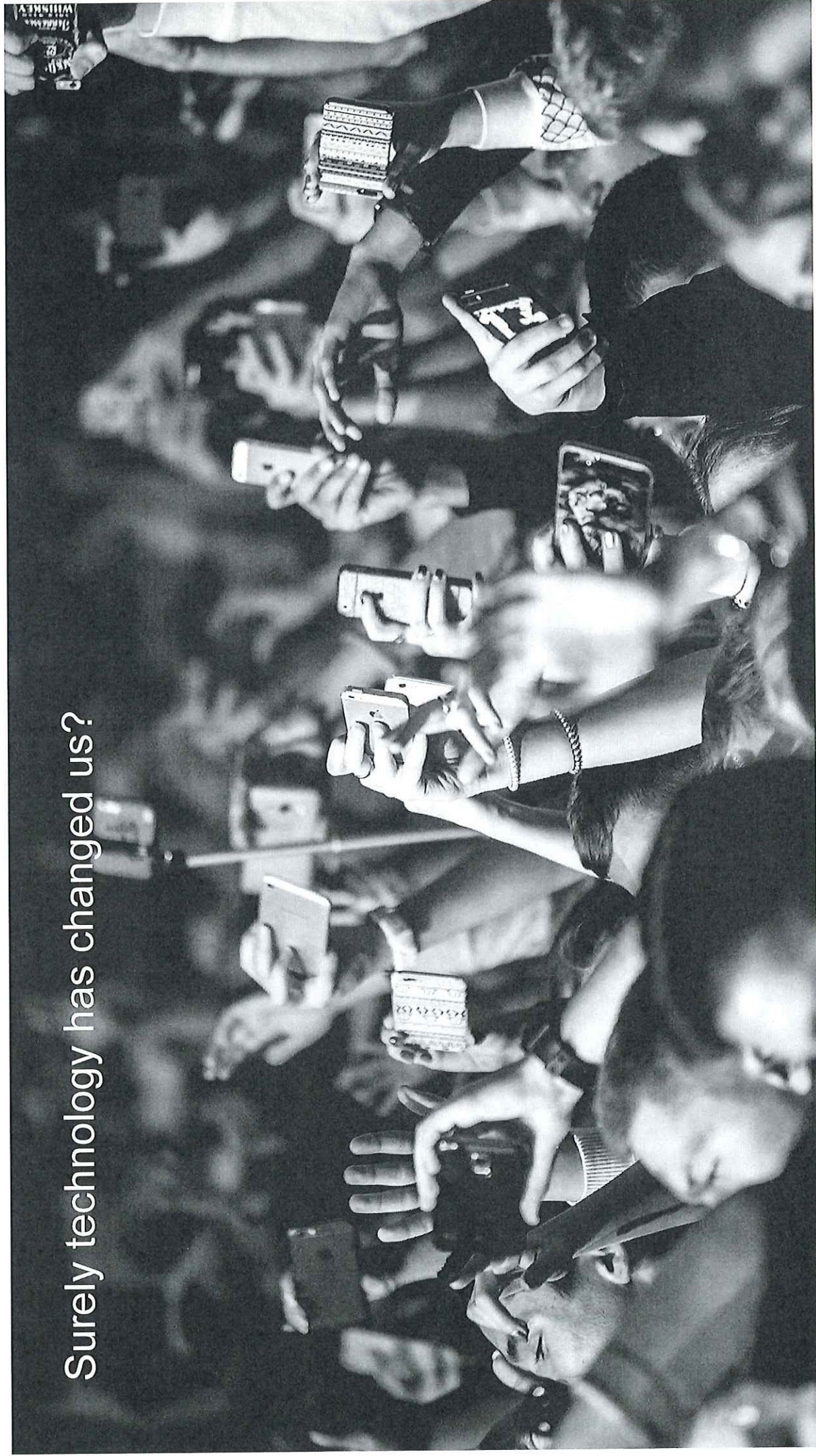




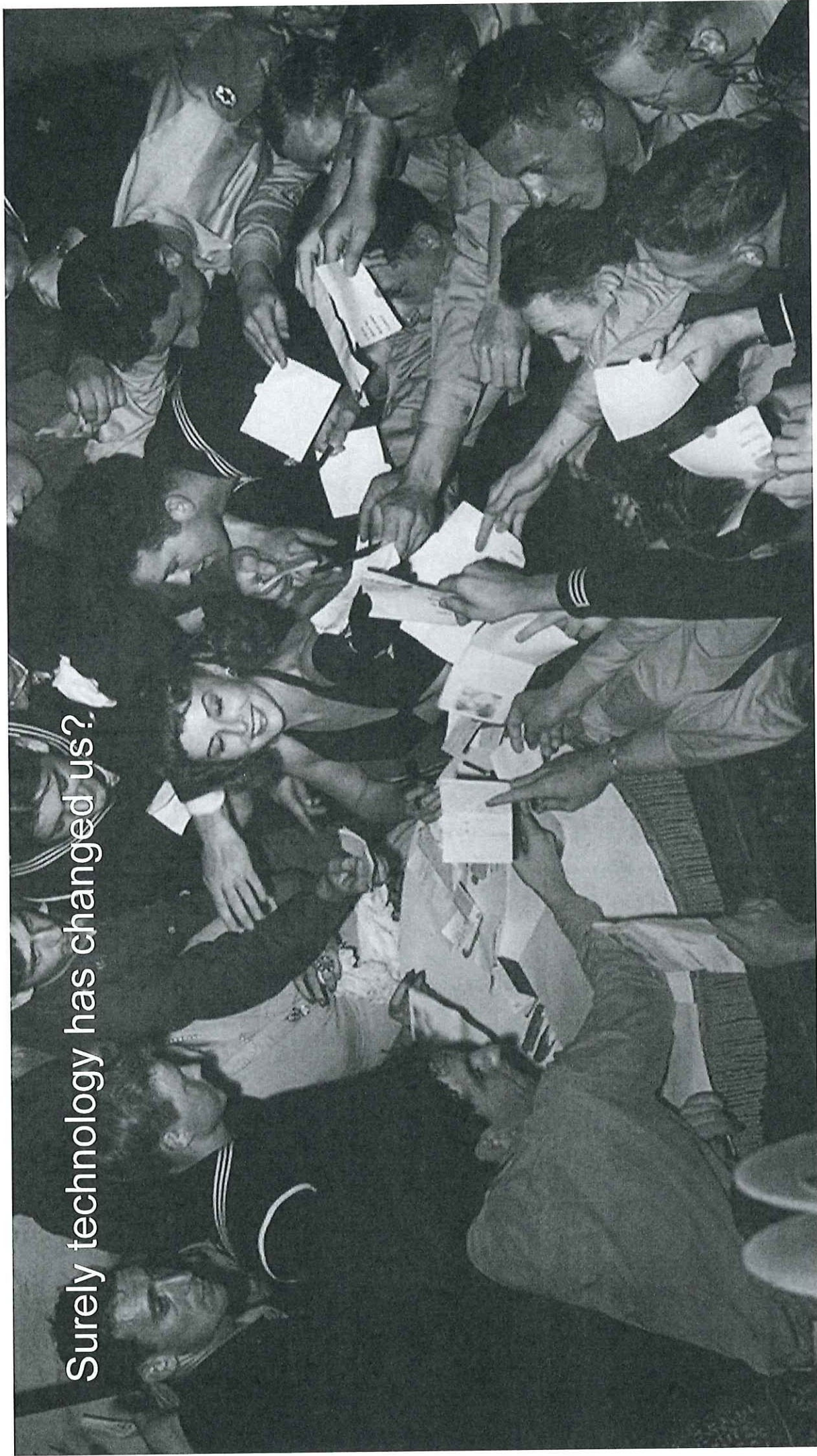


17 x 24 – ?

Surely technology has changed us?



Surely technology has changed us?



"It took millions of years for man's instincts to develop. It will take millions more for them to even vary. It is fashionable to talk about changing man.

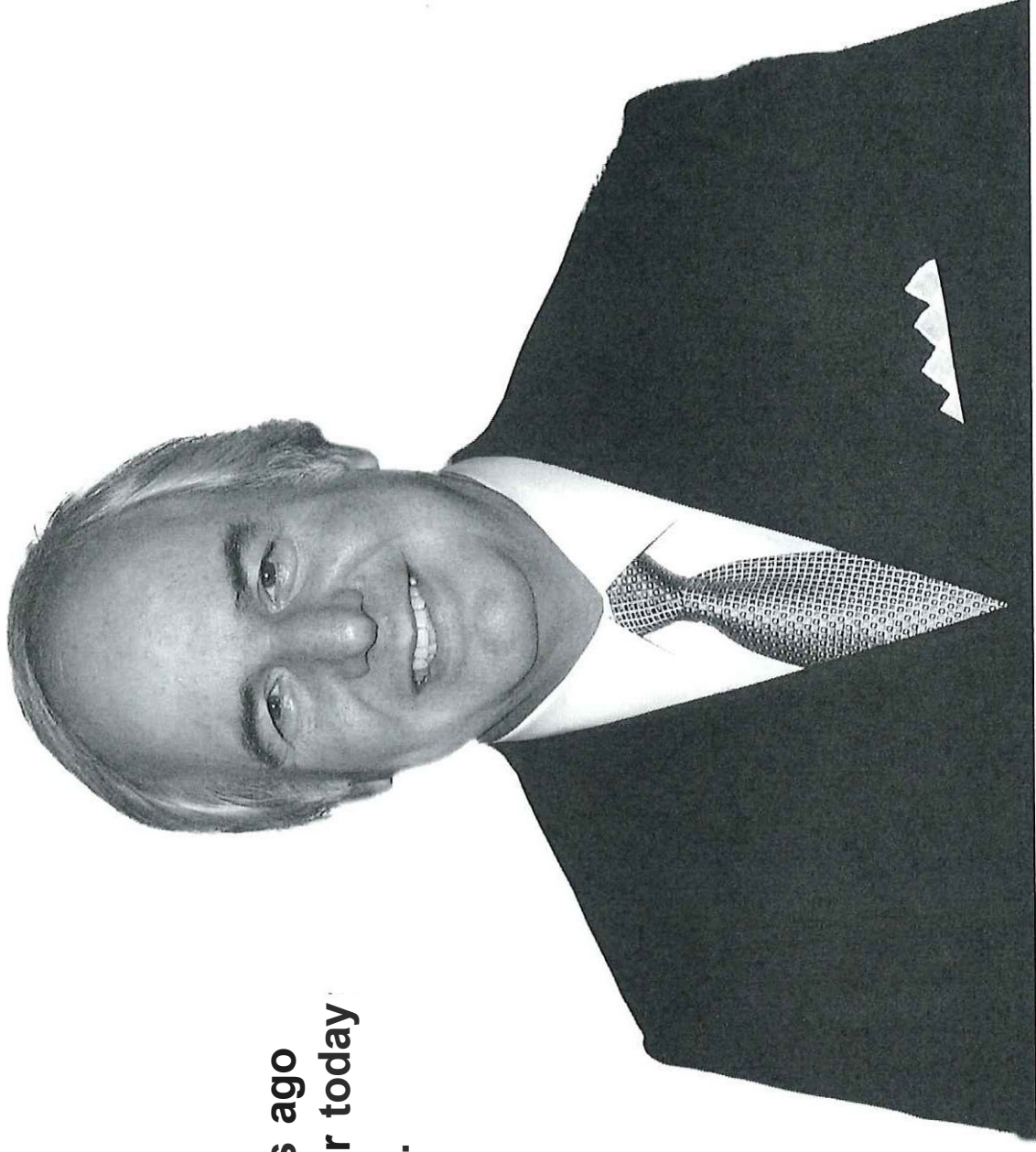
A communicator must be concerned with unchanging man, with his obsessive drive to survive, to be admired, to succeed, to love, to take care of his own."

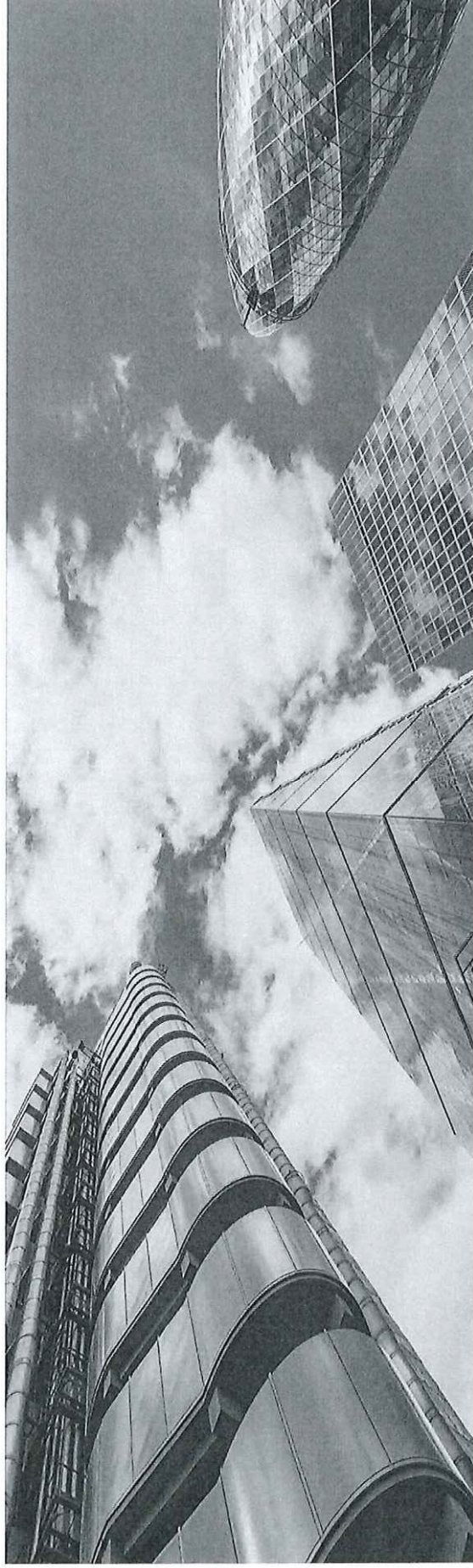
-Bill Bernbach



**"What I did was almost 50 years ago
and it's about 4,000 times easier today
to con people than when I did it.
Technology breeds crime."**

-Frank Abagnale





Health Claims Forum

PROTECTION INSURANCE & WAR-RELATED ISSUES

Helen Tilley, Partner

helen.tilley@cpblaw.com

www.cpblaw.com



11 July 2022

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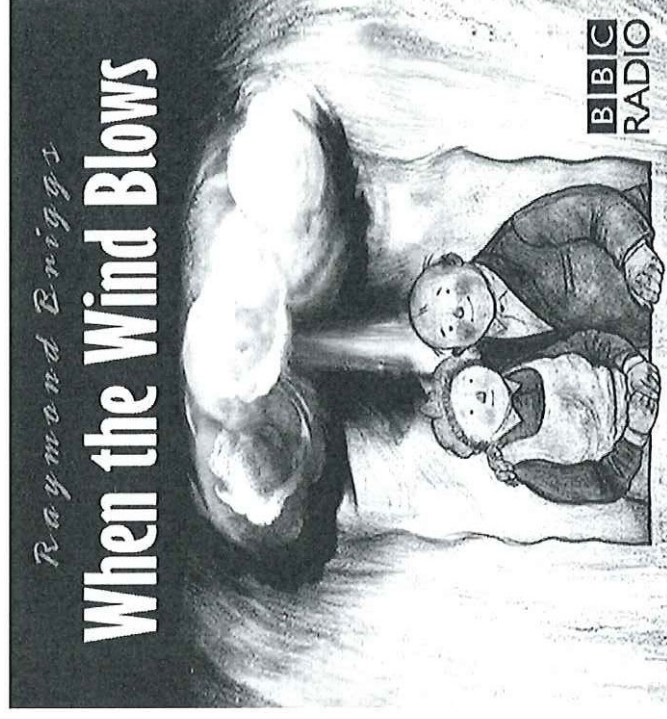
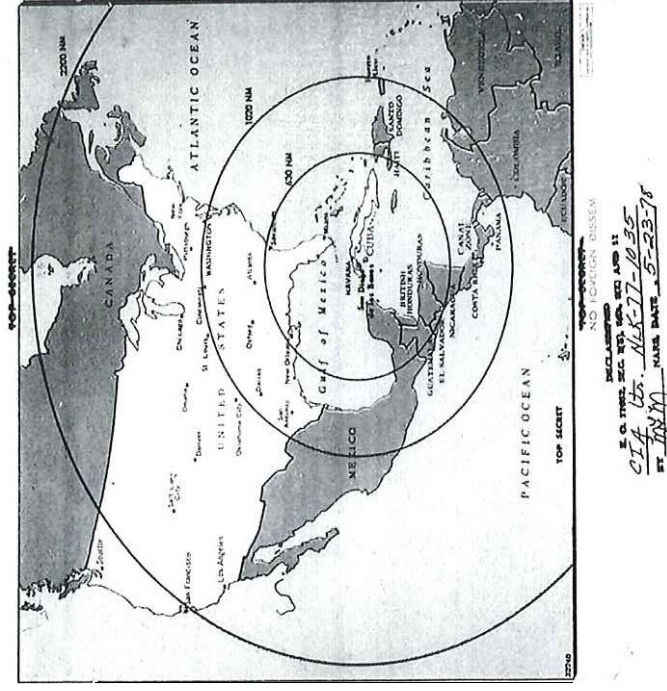
Bailey

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- No liability can be accepted by us for any action taken or not as a result of this information.

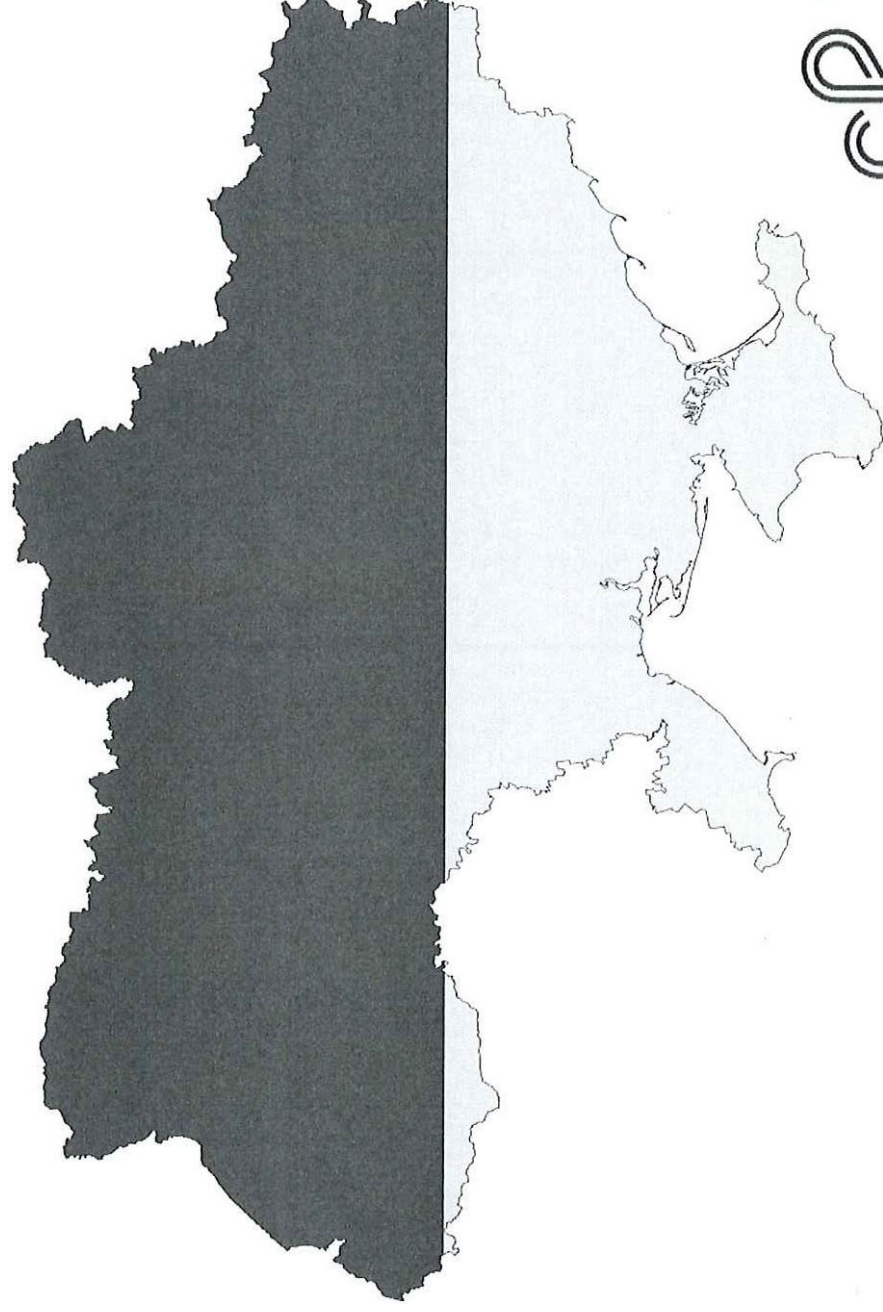


Do you remember this?



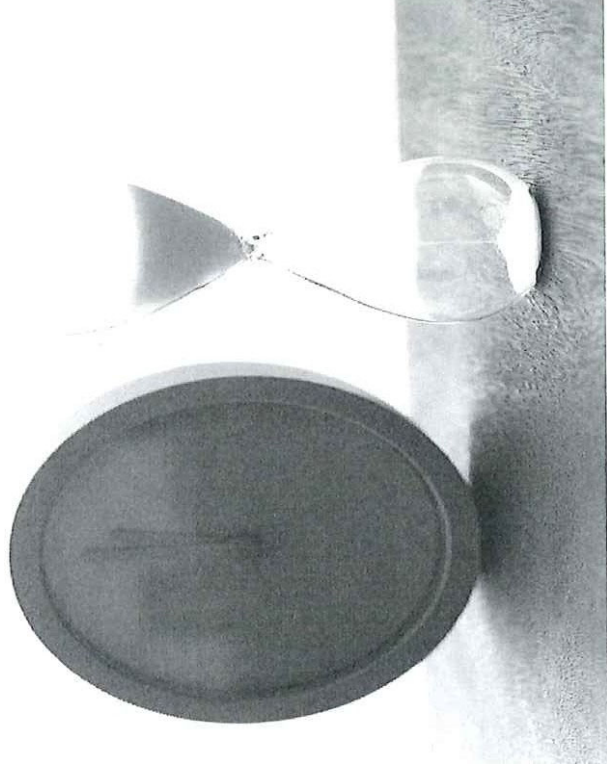
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The 30-year holiday from history is over...



Objectives for Today

- How does war/hostilities impact protection insurance?
- What is the risk appetite generally, taking into account aggregation?
- Is a 're-assessment' of the nature of risk and exposure to that risk due?
- Are our contract wordings fit for purpose?
- Are 'state-sponsored' cyberattacks modern forms of hostile methods in the modern age, rather than traditional warfare?
- What is the day-to-day effect of sanctions?



The build up to 24/2/2022

Sanctions were not new

Between the 2014 Russian annexation of Crimea and the invasion of 24/2/2022, sanctions impacting **183 people** and **53 entities** had been imposed.

AND now a 'nothing off the table' approach to sanctions relating to Russia

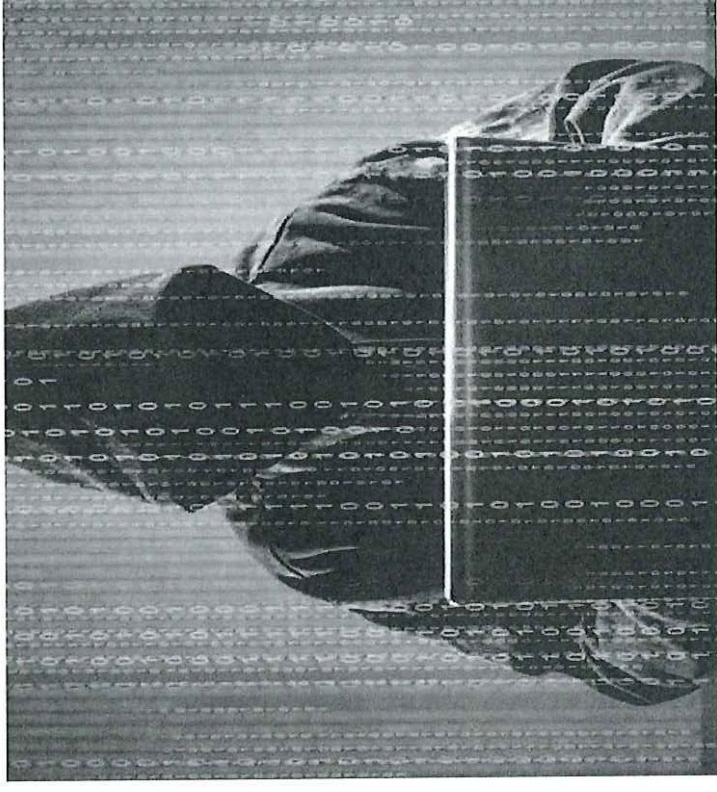
Cyber attacks were not new

Cyber threats had been increasing (e.g. 'NotPetya' malware of accounting software in 2017, which had global effects)

BUT now Governments have warned of increased threat including to financial institutions and healthcare providers

Russian nationals dominated the London Commercial Court cases - (second largest group of litigants in 2021)

Marine risks -Joint War Committee updated Listed Areas from mid 2/2022 (where vessel owners required to notify underwriters due to perceived enhanced risk)



What do the current 'hostilities' and the sanctions regime mean day-to-day?

All

Existing cover:

Prevented from making claim payment (is progress 'paused')?

Is a licence required e.g. to make a payment?

Has there been a qualifying misrepresentation (at application stage)?

Anti-selection? Are proposal qu's still robust?

Has there been a material change in the risk?

Indirect effects:

Cyberattack on healthcare system preventing/slowing Tx (i.e. over 81% of healthcare organisations have faced a ransomware attack) UK

Will existing policyholders cancel cover or reduce cover due to 'cost of living' crisis?

Life/ Critical illness

Sudden cardiac death - examine trigger?

Blunt cranial trauma - cause?

Poisoning?

IP

Has there been a material change in circs with GIP?

Should there now be wider/extra questions on renewal (re occ. / location/ travel plans)?

Does a prohibition to travel prevent insured person returning to region required by insurance contract?



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Appetite to Provide Cover

Designed with war / terrorism in mind:

Political risk & trade credit

Could be willing to insure in war / terrorism context:

- International transport e.g. marine/ aviation/ cargo
- Life/ Protection*
- Travel (terrorism cover)*

**consider with aggregate limits*

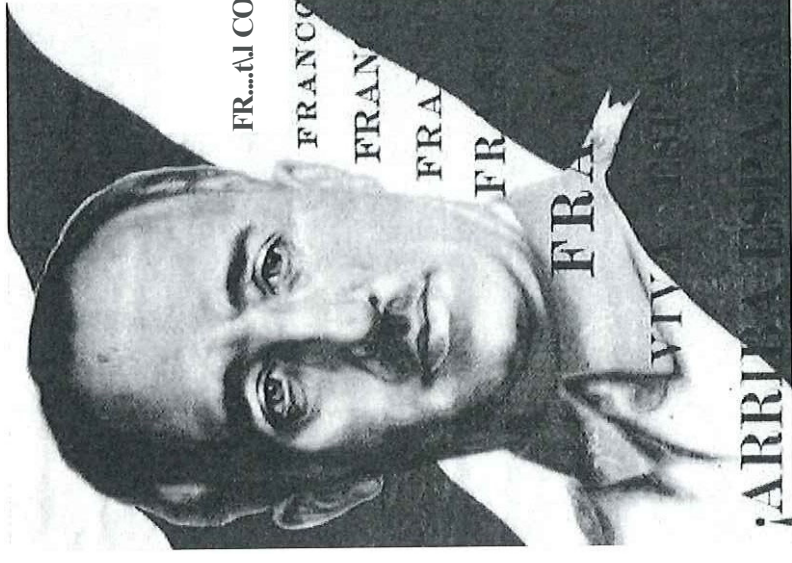
Typically expressly exclude cover an unwilling to cover:

'All risks' property



Development of War Exclusions

- Exclusions protect against insurer insolvency due to potential impact of war related risks on a wide range of insured sectors and/or types of insurance cover, limiting ability to share the risk in usual way
- Usual methods of calculating risk don't apply during wartime
- Main war/ hostilities exclusion (NMA 464) originated post 1930's Spanish Civil War because long-range weapons could extensively damage civilian towns
- In the 21st Century do the techniques of 'war/ hostilities' include cyberattacks, rather than traditional weapons/ warfare?




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Insurance wordings - fit for purpose?

RACE CLAUSE

Radioactive Contamination

Exclusion

Originated due to concerns of radiation escape from **nuclear power stations.**

Extended since the S0's due to post 9/11

Updated post 9/11

To take account of growing concern of separate risk of **terrorism**. 9/11 was not regarded as war, but it was terrorism (which was not typically excluded at that time).

FIVE POWERS CLAUSE

In typical property cover

¹¹This insurance excludes loss damage liability or expense arising from the outbreak of war (whether there be a declaration of war or not) between any of the following: United Kingdom, USSR, United State, Fe eration, n ma, ...

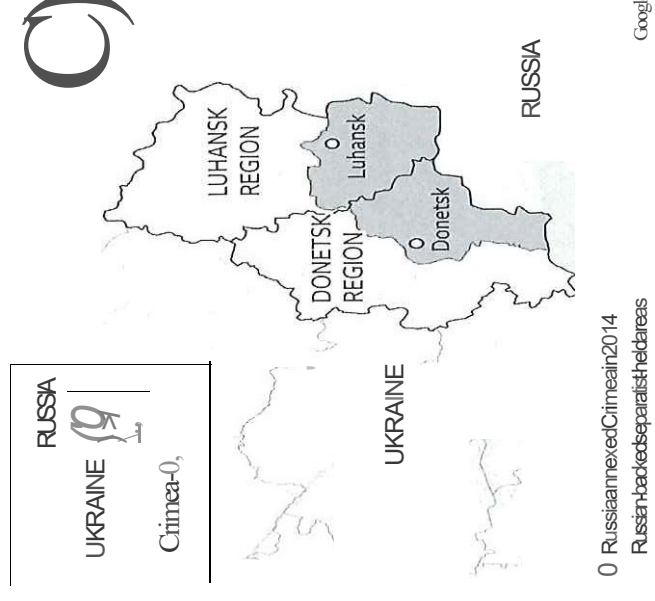
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CL370 extension to RACE exclusion due to concern about use of **electromagnetic pulse weapons in an attack**, which could disable electrical equipment in radius of explosion site (including computers)

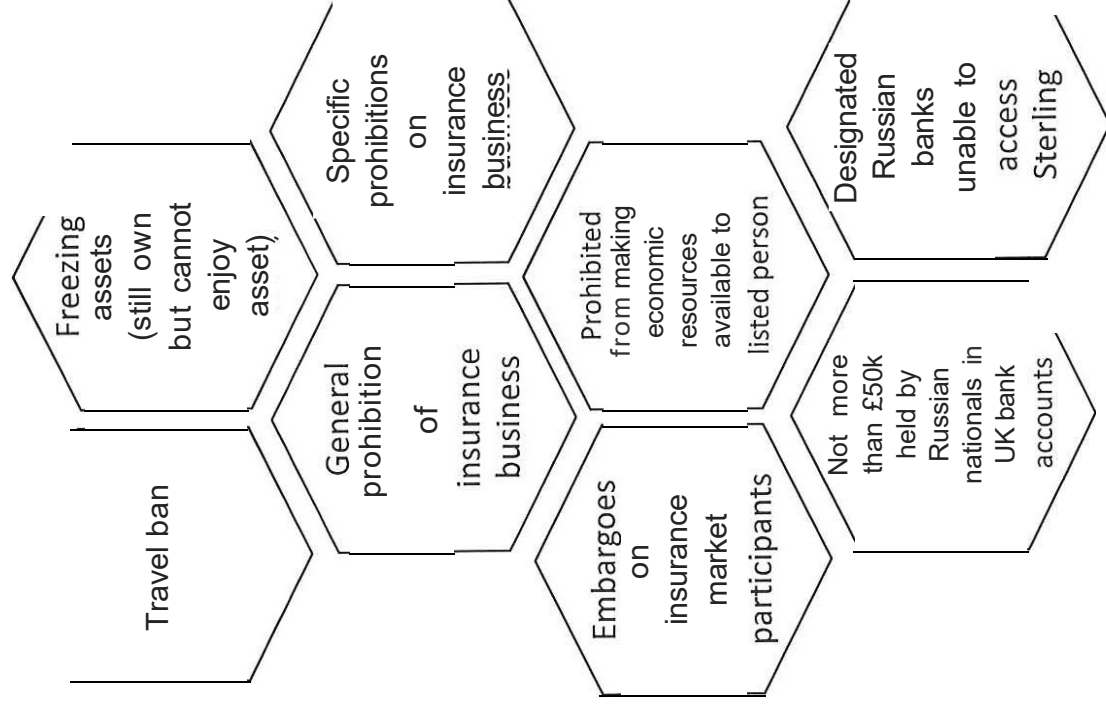
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UK Sanctions - a foreign policy tool

- Between the **2014 Annexation of Crimea** and the invasion, 183 people & 53 entities connected with Russia had already been sanctioned
- **Sanctions and Anti-Money Laundering Act 2018** ('Sanctions Act') - The post-Brexit solution. UK has >35 thematic and country-specific sanctions regimes
- **Magnitsky Sanctions** (re global human rights sanctions named in memory of deceased whistleblower tax lawyer re corruption)
 - o 7/2020 - 25 Russian officials
 - o 12/2020 - 3 Chechen officials & 1 entity (re persecution against Chechnya LGBT+ community)
- **Chemical Weapons Sanctions Regime** - 1/2019 - due to 2018 Salisbury nerve agent & 8/2020 Novichok chemical attack against opposition leader Alexei Navalny
- **Cyber Sanctions Regime** - Russian military intelligence officers and units of GRU
- **Anti-corruption Sanctions Regime** - 4/2021 - 12 Russian nationals



Types of Sanctions



Sanctions should be at the forefront of corporate compliance programmes

Breach of Financial Sanctions

Policing and Crime Act 2017

Imprisonment:

Max sentence has increased from 2 to 7 years

Fines (against individual and/or corporate entity) introduced under s.146 Policing and Crime Act 2017

Max fine flmillion or 50% of est. value of funds/economic resource, whichever is higher

Referral to regulator (FCA): incl SIMR

Criminal Offence Test: "*knew or had reasonable cause to suspect*"

Case Example:

Standard Chartered Bank fined £20.47m in 3/2020 for unintentionally breaching sanctions by lending £97.4M to a Turkish bank (which was majority owned by a Russian sanctioned bank) in good faith thinking this fell within an exemption



Sanctions Searches

(But not a substitute for due diligence)

UK Sanctions List and wider HM Treasury's Office of Financial Sanctions Implementation (OFSI) Consolidated List

UK enforcement by: OFSI (which can also refer to regulators e.g. FCA)

<https://www.gov.uk/government/publications/the-uk-sanctions-list>

<https://www.gov.uk/government/publications/financial-sanctions-consolidated-list-of-targets/consolidated-list-of-targets>

N.B. has the individual/ entity asked to vary or revoke a UK sanctions designation?

EU Sanctions create legal obligations for all EU citizens, operators and any business conducting within the EU

EU enforcement mainly lies with Member States

N.B. Violations can be reported anonymously via the EU Sanctions Whistleblower tool <https://eusanctions.integrityline.com>

US Sanctions apply to all US persons, wherever located, which includes subsidiaries of US Companies (including London based branch of US headquartered insurer)

US enforcement by: Office of Foreign Assets Control (OFAC)

<https://sanctionssearch.ofac.treas.gov/>



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International Coordination

- International coordination of (non-UN) sanctions is important in context of Ukraine/Russia because **UN Security Council resolutions** stifled by ability of Russia to veto and block vote!
- **Economic Crime (Transparency and Enforcement) Act 2022** allows UK to immediately designate individuals and entities already sanctioned by the EU, US and other allies

- **BUT** there remain **inconsistencies** between different jurisdictions, adding to complexity of sanctions compliance (e.g. Roman Abramovich)

Western-led sanctions not supported by:

China

and

India

Iran

USA

EU

Israel

Turkey (Non-Aligned Movement State) & Serbia (EU candidate country) have declined to implement sanctions)

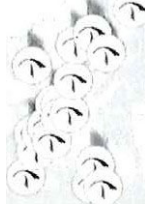


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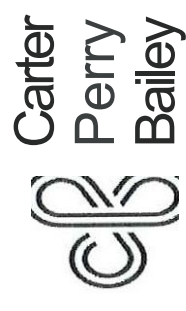
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War & Terrorism Risks Team



Helen Tilley
PARTNER



Stephen Carter
MANAGING PARTNER



Paul Bugden
CONSULTANT



Robert Harrison
CONSULTANT



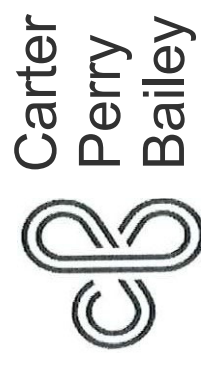
Samantha Zaozirny
SENIOR ASSOCIATE



Dean De Cesare
SENIOR ASSOCIATE

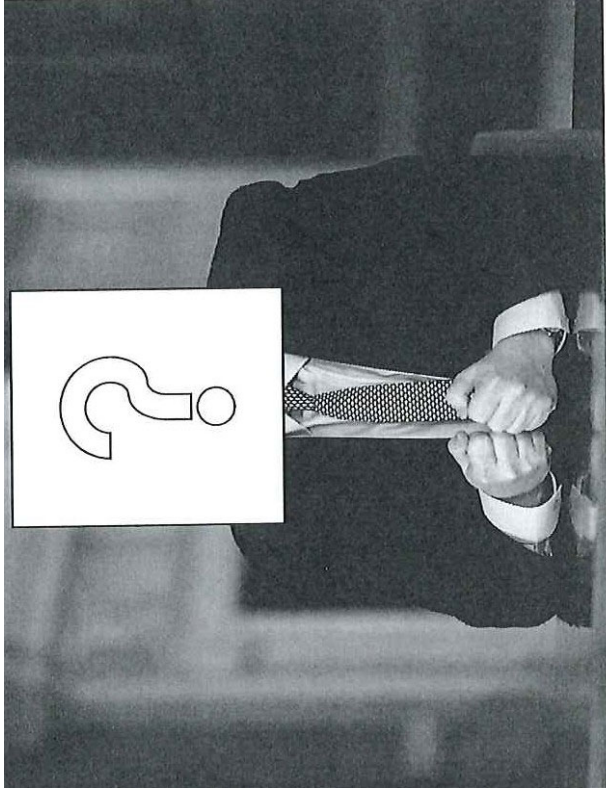


Lisbeth Poulsen
SOLICITOR



What if we go to war?

- The effect of war on contracts (generally - not just insurance contracts) is that 'aliens' become enemies
- You cannot pay insurance monies to an 'enemy' during wartime - due to the **Trading with the Enemy Act 1939** - which will likely be adapted/modernised, if the UK goes to war
- Insurance contract is not automatically dissolved - if the insured risk remains **legal** then payment of the claim is 'paused' (i.e. suspended until the end of the war) and the insured (enemy) needs to continue to comply with their contractual obligations
- If insured risk becomes **illegal** then the enemy insured cannot claim at all



Closing Comments

- Ensure sanctions compliance is sufficiently resourced in anticipation of more proactive enforcement
- Review aggregation exposure scenarios
- Are greater application & renewal questions in order?
- Will there be greater use of policy exclusions going forward? If so, consider signposting to those with higher risk appetite & access to local security/evacuation firms in known 'hotspots'
- Remind customers to consider putting their Life cover in Trust (or update beneficiary nominations)

Scandinavia

Kazakhstan

Lebanon

USA

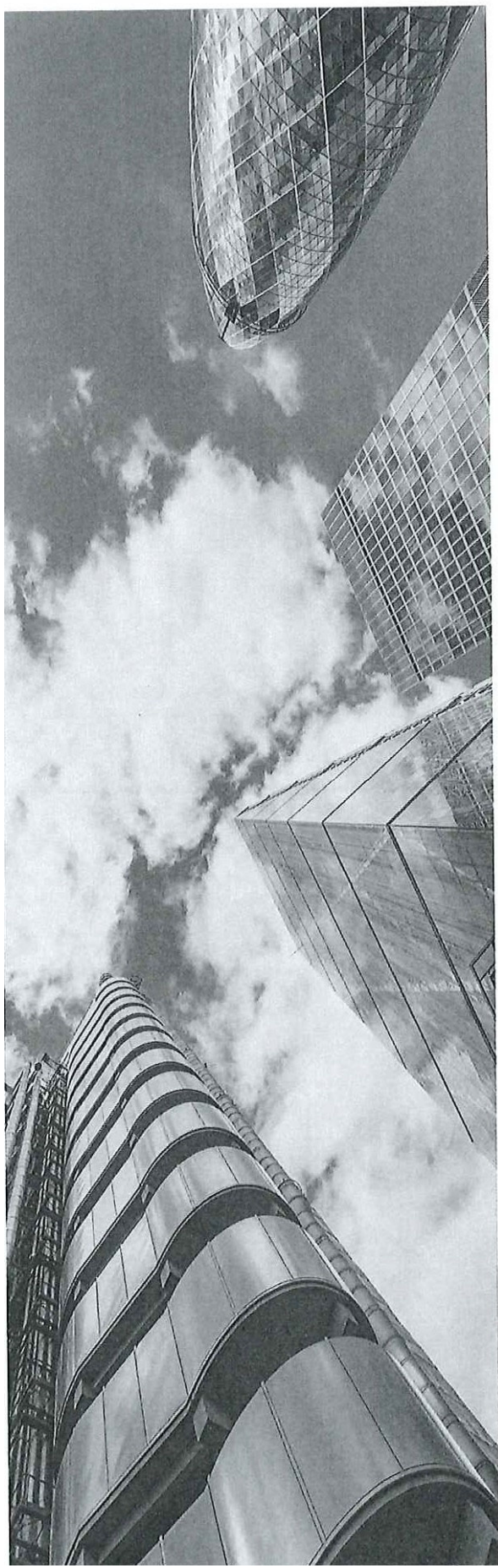
Syria

Colombia

Iran Italy

Mali

UAE



Any Questions?

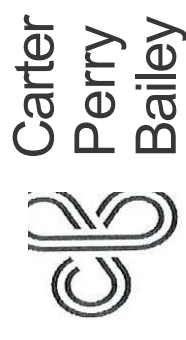
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Further Information

Recent related CPB article: *'It's war - But not as we know it?'* <https://lnkd.in/eS8M8mkY>

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<https://www.linkedin.com/company/institute-for-the-study-of-war> {Think Tank}
<https://uk.linkedin.com/in/robin-horsfall-52243220> {Insight from ex-SAS soldier)

Assistance for overseas personnel/ emergency evacuations:

Dan Kaine {InherentRisks.com)

Crisis24.com

Northcottglobalsolutions.com

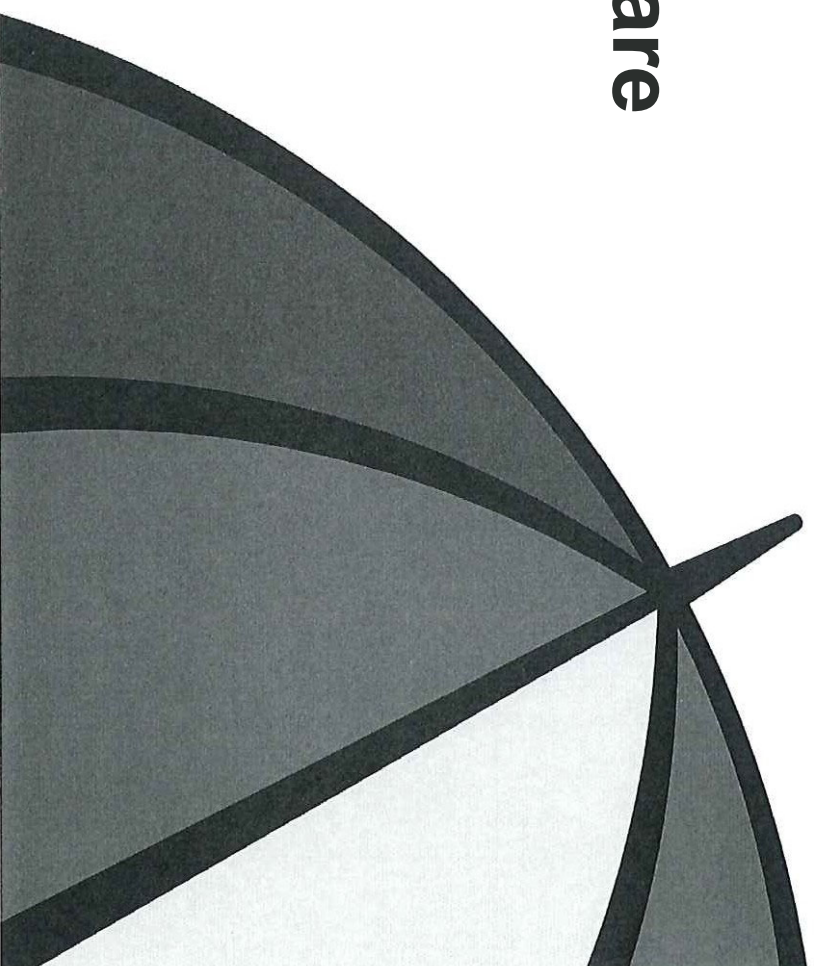


Developments in Digital Care

Dr Tarun Gupta

Medical Officer, Group Income Protection, L&G

July 2022



Introduction

The Pandemic has accelerated the adoption and acceptance of new models of care across the NHS and private sector, including 'digital'

Digital health: use of digital technologies to improve access to healthcare, improve the quality of health outcomes and to enhance the health and wellbeing of the population

- Changes across both primary care and secondary care
- Opportunity to improve access, patient engagement and health outcomes, but also some challenges
- Implications for the claims sector - now and going forward

Key areas

Digital empowerment

- Adoption of alternatives to face-to-face consults e.g. video
- Access to personal health records
- Improving self-help and online care - apps, websites, wearables and home devices

Digital culture

- Digital innovation to support quality improvement
- Development of digital 'pathways of care'
- Mobile and flexible working for staff - impact on recruitment / retention

Digital foundation

- Interoperability across systems
- Data protection
- Using 'big data' to aid prevention, predict need and improve health (whole population insight)

Changes in primary care

- **Access and choice:** far greater focus on online self-care resources, online 'triage', online consultations (phone and video)
 - Easier follow-ups e.g. medication reviews
 - Less suitable for children, complex cases
- **Personalisation and ownership:** book and change appointments online, book with different services online, suite of apps to prevent ill-health, NHS app evolving to provide access to GP health record
- **Staffing and capacity:** engaging a clinical workforce that can work partially remotely helps address recruitment and retention issues as staff can be based further afield but also means less 'continuity' across GPs in a practice/ PCN

Changing role of the 'GP' and 'medical notes'

Far greater focus on a multi-disciplinary team in primary care, e.g. mental health nurse, nurse specialists alongside GPs - there is likely to be less 'GP' contact for patients going forward and an assumption that a single GP/ the 'GP' knows the most about a given patient, depending on the condition

Greater role for employer-led and private 'digital GPs'

- Expanding the offering from just 'acute' / 'reactive' to 'preventive' care
- Increased focus on chronic disease management
- Some are using 'big data' to offer 'predictive' offerings re new 'screening' packages and consultations designed around picking up conditions earlier, outside NHS screening

Digital divide

- Not all patients prefer or are able to access healthcare this way
- Is there a risk of leaving some people behind?

A GP & GP Appraiser's Perspective

- **Remote consults** are more challenging for those less able to articulate their problem concisely and accurately, resulting in more 'risk-taking' for the GP - they can also be more time-consuming and exhausting - it can be more difficult to elicit the 'ICE' (ideas, concerns and expectations)

Several GPs report positive benefits of virtual consults

- less time spent on home visits, able to upload photos of dermatological conditions to notes/ share with specialist for advice and guidance etc

This has been adopted as part of 'BAU' in most surgeries

- **Improved access** is not always better or clinically necessary - several conditions self-resolve within 48 hours for instance and medical input is not always required

Many GPs have sought and received additional training in 'triage'

- **Flexibility** around clinic hours has led to the role becoming more enjoyable v previous rigidity although there can be less 'camaraderie' and scope to discuss tricky cases with colleagues

Changes in secondary care

- **Access:** online consultations (phone and video) being used increasingly for follow-up, although often for initial assessment too, depending on the specialty

- **Personalisation:** patients able to share data with specialists and 'own' their care more

- **Staffing and capacity:** engaging a clinical workforce that can work partially remotely helps address recruitment and retention issues as staff can be based further afield

Changes

Freeing up more time for care: better access to 'advice and guidance' for primary care via 'digital' should reduce the number of people who need further specialist assessment

With improved data sharing and access to patient records, specialists are able to take a more 'holistic' view and reduce duplication of bloods etc

Improved data collection and triangulation to provide greater insight with which to make decisions

Impact on insurers - innovation around claims management

Preventive

- More 'ownership' and interest in apps, wearables etc could result in earlier detection of cardiovascular disease, diabetes etc
- Likely some screening e.g. NHS Health Check for those over 40 years has been missed/delayed during the last 2 years due to the Pandemic

Reactive

- Improved access could mean certain 'acute' issues are less likely to evolve to become 'chronic' due to earlier intervention
- Chronic disease management care is likely to become more fragmented, with several healthcare professionals in one organisation and / or across organisations involved in that person's care - a shift away from 'one' owner of ongoing treatment, response to therapy etc

GP contract 2022/2023

- NHS England has removed the contractual requirement that at least 25% of appointments are available for online booking. There will be a 'more targeted requirement' that all appointments which do not require triage are able to be booked online, as well as in person or via the telephone
- All practices to offer the full range of 'routine' services such as screening, vaccinations and health on Saturdays from 9am to 5pm
- No new additional indicators will be added to QOF when the temporary income protection arrangements come to an end in March 2022. Quality Improvement (QI) modules for 2022/23 will focus on optimising patients' access to general practice and prescription drug dependency
- There will be another £280m available for PCNs to recruit additional staff under the additional roles reimbursement scheme
- There will be a doubling of mental health practitioners roles to support people with complex mental health needs, plus additional flexibility to help support recruitment to these roles

Quality & Outcomes Framework (QoF) - update

- The size of QOF has stayed the same at 635 points
- The value of a QOF point in 2022/23 will be £207.56 and the national average practice population figure will be 9,374. These changes cancel out each other out so there is no real-term QOF uplift
- There are no changes to QOF indicators or payment thresholds for 2022/23
- Asthma and heart failure lists to be reviewed to ensure patients diagnosed within the last six months have coded investigations
- Focus on monitoring indicators that are time sensitive and require follow-ups after a set time period after a diagnosis - mainly across depression, cancer and diabetes
- Physical health checks for people with serious mental illness (c 1% of the population) - defined as people with diagnoses of bipolar, psychosis or schizophrenia

Thank you

Any questions?